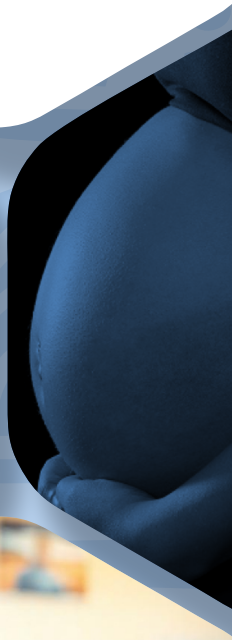
intersex
NIGERIA
CENTER FOR HEALTHCARE DEVELOPMENT & YOUTH EMPOWERMENT



Medical Practices and Experiences Surrounding Intersex Births in Nigerian Hospitals

By Intersex Nigeria
(Center for Healthcare Development and Youth Empowerment)

Copyright©: Intersex Nigeria

Intersex Nigeria
(Center for Healthcare Development and Youth Empowerment)

Contact Information

Intersex Nigeria (Center for Healthcare Development and Youth Empowerment)

Lagos and Abuja, Nigeria

info@intersexnigeria.org

+2349129057794

Visit our website

www.intersexnigeria.org

Social Media

@Intersex_nigeria_

@IntersexNigeria

@IntersexNigeria

Intersex Nigeria

Published by: Intersex Nigeria
Copyright: Intersex Nigeria, 2024

Publication Date:
December, 2024

© Intersex Nigeria

Table of Contents

I	**Table of Content**	**I**
II	**Forward**	**II**
III	**Acknowledgement**	**III**
IV	**Executive Summary**	**IV**
1.0	**Introduction**	**01**
1.1	Background	01
1.2	Objectives	02
2.0	**Understanding Intersex Biological and Medical Perspectives**	**04**
2.1	Psychological and Social Impacts of Medical Interventions	04
2.2	Human Rights Violations and Legal Protections	04
2.3	Healthcare System and Practices	06
2.4	Review of Literature on Intersex Studies in Africa	06
2.4.1	Prevalence of Surgical intervention/(IGM)	07
2.5	Overview of Intersex Studies in Nigeria	07
3.0.	**Methodology**	**11**
3.1	Mapping out Hospitals and Medical Practitioners	11
3.2	Data Collection and Analysis	11
4.0	Findings and Discussion	14
4.1	Demography of the respondent	14
4.2	Regional Representation and Specialties	14
4.3	Awareness and Knowledge of Intersex	16
4.4	Prevalence of Intersex Births	17
4.5	Clinical Evaluation and Treatment on Intersex Persons	18
4.6	Attitudes and Practices of Healthcare Providers	24
4.7	Societal and Cultural Influences on Intersex Treatment	25
4.8	Discussion of quantitative findings	25
4.9.	Discussion of interview findings	26
4.9.1.	Challenges and ethical considerations involved in treating intersex individuals	27
4.9.2.	Experiences with intersex in terms of diagnosis and treatment approach in Nigeria	28
4.9.3.	Impact of cultural and societal beliefs in the management and treatment of intersex in Nigeria	29
4.10.	Effects of Surgical Intervention on Intersex Persons	30
5.0.	**Conclusion**	**32**
5.1	Recommendation	32
	References	**34**
	List of Charts and Tables	**36**
	Appendices	**37**

Foreword

The field of medical science is ever-evolving, yet some areas remain underexplored, often due to societal stigmas, interest from medical professionals and no priority. Among these areas is the field of intersex, a subject that intertwines complex biological, psychological, and social dimensions. This research aims to shed light on these complexities, exploring the occurrence of intersex birth, medical practices, and social challenges faced by intersex individuals. **The reason for this study arises from a pressing need to address the gaps in knowledge and the inconsistencies in care of intersex individuals in medical settings.**

Despite the growing awareness of intersex issues globally, much remains to be done, particularly in regions **where cultural and religious beliefs significantly influence medical practices and societal attitudes.**

Through quantitative and qualitative data collection and analysis, this research delves into the occurrence of intersex birth, prevalence of Intersex Genital Mutilation, the diagnostic criteria and methods employed by healthcare professionals, and the treatment options available. It also highlights the profound impact of medical interventions both positive and negative on the lives of intersex individuals, as well as the broader societal implications. This study is not a scholarly endeavour, but a call to action. It urges healthcare providers, policymakers, and society at large to recognise the unique needs of intersex individuals and to advocate for more informed, sensitive, and ethical medical practices. **The findings presented here are a step toward promoting greater understanding and support for intersex people, ensuring that their rights and dignity are upheld.**

It is our hope that this research will serve as a valuable resource for healthcare professionals, scholars, advocates, stakeholders and donor agencies committed to advancing the well-being and rights of intersex people. Let this work inspire ongoing dialogue, education, and reform, ultimately leading to a world where every person, including intersex people is valued and respected for who they are, regardless of their innate biological differences.

Acknowledgement

Intrsex Nigeria (Center for Healthcare Development and Youth Empowerment) extends our heartfelt gratitude to all the healthcare professionals, patients, and families who participated in and contributed to this significant research report.

Your openness and perspectives have been invaluable. We commend the consultants, Dr. Dauda Bathnna and Parton Titus, for their dedication and commitment to seeing this work come to fruition. Their work is a testament to the power of research in driving social change and improving lives. The insights gained from this study will undoubtedly contribute to a more informed and empathetic approach to intersex persons in both medical practice and societal attitudes.

Intersex Nigeria appreciates the unwavering support of CoC Netherlands through the Power of Pride program in Nigeria. We thank you for your intentional support towards ensuring that intersex issues in Nigeria are visible. we could not have achieved this report without your support.

To the entire team at Intersex Nigeria, thank you for your unwavering commitment to ensuring that this project is completed.

Executive Summary

This research work explores the awareness, knowledge, and clinical practices regarding (the term) intersex among healthcare professionals in Nigeria. Through a comprehensive survey of respondents from various regions across the country, the study delves into the frequency of intersex cases, surgical intervention, diagnostic methods, treatment protocols, and the challenges faced by intersex individuals following medical interventions. The aim is to provide a clear understanding of the current state of intersex healthcare in Nigeria, highlighting gaps in knowledge, the prevalence of surgical intervention, and the need for standardized guidelines and training.

The findings reveal that 85% of healthcare professionals had encountered at least one intersex case in the past five years, with a significant number of these professionals observing multiple cases. This underscores the importance of developing appropriate clinical protocols to manage these intersex individuals effectively. Consent for treatment was predominantly given by parents, highlighting ethical considerations regarding decision-making for young intersex individuals. Stigmatization and sexual dysfunction were the most requently reported challenges faced by intersex individuals after treatment, indicating significant psychosocial impacts that need to be addressed in post-treatment care.

One very key recommendation is to create consistent, evidence-based guidelines for the diagnosis and management of intersex across all healthcare facilities.

This research is crucial as it sheds light on the current state of intersex healthcare in Nigeria, a topic that is often under-researched and misunderstood. By identifying gaps in knowledge, inconsistencies in clinical practices, and the challenges faced by intersex individuals, this report provides a foundation for improving healthcare services and outcomes for this marginalized group.

The recommendations offered are aimed at fostering a more inclusive healthcare system, ensuring that intersex individuals receive the respectful and competent care they deserve. As intersex become more recognized within medical and social frameworks, the insights from this study are vital for guiding future policies, clinical practices, and public awareness efforts in Nigeria and beyond.

1.0 Introduction

1.1. Background

In Nigeria, intersex individuals face significant challenges and systemic discrimination due to their innate variations in sex characteristics (chromosomes, gonads, genitalia, hormones, etc), which do not conform to the medical and societal definition of male and female bodies. The Nigerian healthcare system has been largely unresponsive to the unique needs and issues faced by intersex persons. This indifference has led to the enforcement of harmful medical intervention and documentation practices, secrecy surrounding intersex birth, invalidation of healthy variations, unconsented and, unnecessary medical interventions that often result in severe physical, medical and psychological harm.

As a result, intersex persons are subjected to high levels of dehumanization, including isolation, traditional harmful practices, stigmatization, wrongful sex assignment, mob attacks, infanticide, and unnecessary medical (surgical and hormonal therapy) interventions.

Despite experts suggesting that an estimated 0.05% to 1.7% of the global population is born intersex (Esteban C. et.al (2023), there is a significant lack of data on the intersex population in Nigeria. Births of intersex babies often go unrecorded, and reports of violations such as continuous intersex genital mutilation (IGM), public humiliation due to physical sex appearance, and harassment of intersex individuals remain undocumented. This data gap makes it challenging to present comprehensive, evidence-based reports to engage government institutions, policymakers, and influencers in advocacy efforts to end IGM and other human rights violations against intersex persons in Nigeria.

There is a need for a systematic approach in documenting intersex births and occurrences of intersex medical intervention in Nigerian hospitals. This research seeks to address this data gap by collecting and analyzing data on intersex births and medical practices on intersex persons across the six geopolitical zones of Nigeria. By doing so, we aim to create a robust, evidence-based advocacy tool to engage relevant stakeholders and drive policy reform.

1.2. Objectives

The primary objectives of this research are

I. To develop a comprehensive report on the intersex birth rate in at least 50 selected tertiary hospitals across the six geopolitical zones in Nigeria.
II. To document the occurrence and prevalence and practices of non-consensual Surgical intervention (IGM) and other medical interventions on intersex persons (babies, children, and adults) in Nigeria.
III. To highlight the dangers and negative impacts of non-consensual Surgical intervention (IGM) and other medical interventions on intersex persons.
IV. To gather adequate information and facts to inform advocacy efforts aimed at ending IGM and other armful medical interventions in Nigeria.
V. To provide an evidence-based report for engaging the government, community leaders, healthcare providers, and policy influencers in support of intersectional advocacy processes.s that protect the rights and well-being of intersex persons.

2.0. Understanding Intersex Biological and Medical Perspectives

Intersex variations encompass a range of biological variation where an individual's chromosomal, gonadal, or anatomical sex characteristics composition does not fit into the medical and societal definition of male and female bodies. According to Dreger (1998), these variations include Androgen Insensitivity Syndrome (AIS), Congenital Adrenal Hyperplasia (CAH), and Klinefelter Syndrome. Fausto-Sterling (2000) argues that these natural biological variations challenge the binary understanding of sex, emphasizing the complexity and diversity of human biology.

Historically, medical literature has pathologized intersex traits, viewing them as disorders that require correction. Karkazis (2008) discusses how the medical community has traditionally approached intersex with an emphasis on early surgical intervention to align the individual's physical appearance with societal gender norms.

This medicalization often disregards the long-term health and psychological well-being of intersex individuals, prioritizing cosmetic outcomes over holistic care.

2.1. Psychological and Social Impacts of Medical Interventions

Studies have shown the adverse psychological effects of non-consensual genital surgeries on intersex individuals. Tamar-Mattis (2013) stated the long-term trauma, including chronic pain, loss of sexual function, and significant psychological distress, that many intersex people experience as a result of these interventions.

This trauma is compounded by feelings of betrayal and a lack of bodily autonomy, as these surgeries are often performed during infancy or childhood without the individual's consent.

Social stigmatization and identity struggles of intersex individuals is well-documented in sociological literature. Davis (2015) mentioned how societal expectations and rigid gender binaries contribute to the marginalization and discrimination of intersex persons.

The stigmatization extends to families, who may feel compelled to hide or "fix" their child's intersex traits to avoid social ostracism. These pressures can lead to significant identity struggles and internalized stigma for intersex individuals, affecting their mental health and social integration.

2.2. Human Rights Violations and Legal Protections

The legal recognition and protection of intersex individuals' rights is a critical issue.

Carpenter (2016) argues that the lack of specific legal frameworks to protect intersex persons results in widespread human rights violations, including non-consensual medical interventions and discrimination. International human rights bodies, such as the United Nations, have called for an end to practices like Intersex Genital Mutilation (IGM) and have urged countries to implement policies that protect the bodily autonomy and dignity of intersex individuals. Resolution 552 recalls article 2 of the Charter which prohibits any form of discrimination. In this regard, states must take the necessary measures to ensure that the rights of people are guaranteed and protected, irrespective of their race, colour, nationality, citizenship, ethnicity, profession, political opinions, sex, sex characteristics, sexual orientation, gender identity, gender expression or any other factor that could lead to discrimination against them. The interpretation of article 2 of the African Charter is open ended and inclusive, and aims at offering the maximum protection to all Africans, hence the inclusion of sex characteristics, gender and sexual orientation as prohibited grounds of unfair discrimination. By implication, states must prohibit discrimination on the basis of intersex traits, characteristics or status in all spheres of life and social institutions including education, health care, employment, competitive sports, access to public services, and address such discrimination through relevant anti discrimination initiatives.

The Resolution 552 of the African Charter on Human and Peoples Rights (ACHPR) affirms that intersex persons in Africa are equal before the law and therefore should be treated with dignity, and enjoy equal
protection under the law.

Similarly, the United Nations Human Rights Council, through Resolution 55, Combating Discrimination, Violence, and Harmful Practices Against Intersex Persons, encourages States, in collaboration with relevant international and regional organizations, to enhance efforts to combat discrimination, violence, and harmful practices against persons with innate variations in sex characteristics.

The resolution calls for addressing root causes, such as stereotypes, the spread of misconceptions and inaccurate information, stigma, and taboo. It also urges States to work toward ensuring the highest attainable standard of physical and mental health for persons with innate variations in sex characteristics.

The Special Rapporteur on Torture highlights that children who are born with atypical sex characteristics are often subject to irreversible sex assignment, involuntary sterilization, involuntary genital normalizing surgery, performed without their informed consent, or that of their parents, "in an attempt to fix their sex" …77, leaving them with permanent, irreversible infertility and causing severe mental suffering.

The special rapporteur calls upon all States to repeal any law allowing intrusive and irreversible treatments, including forced genital-normalizing surgery, involuntary sterilization, unethical experimentation, medical display, "reparative therapies" or

"conversion therapies", when enforced or administered without the free and informed consent of the person concerned. This report also calls upon states to outlaw forced or coerced sterilization in all circumstances and provide special protection to individuals belonging to marginalized groups.

It is important to note that advocacy efforts by intersex organizations is pivotal in raising awareness and pushing for legal reforms. The role of intersex activists in challenging harmful medical practices and advocating for legal recognition and protection is very important Holmes (2011).

These efforts have led to incremental changes in some countries, where legal reforms have been introduced to prevent non-consensual surgeries and to recognize intersex individuals' rights to self-identify.

2.3. Healthcare System and Practices

The healthcare system's approach to intersex variations is often hindered by a lack of training and ethical guidelines for healthcare providers. Garland and Slokenberga (2020) emphasize the need for comprehensive education and training programs to equip medical practitioners with the knowledge and sensitivity required to provide appropriate care for intersex individuals. They also call for the development of ethical guidelines that prioritize informed consent and the long-term well-being of intersex people.

Intersex individuals face numerous barriers in accessing healthcare, including discrimination, financial constraints, and lack of specialized services. Creighton (2014) highlights how these barriers contribute to health disparities and poor health outcomes for intersex persons.

The literature suggests that addressing these barriers requires systemic changes in healthcare policies, including ensuring insurance coverage for intersex-related care and creating supportive environments in medical settings.

2.4. Review of Literature on Intersex Studies in Africa

Intersex people are often misunderstood and stigmatized within many African cultures.

Amadiume (1987) and Oyěwùmí (1997) discuss how traditional African societies have historically recognized diverse sexes, but colonial and post-colonial influences havereinforced rigid sex categorization. These influences have contributed to the stigmatization and marginalization of intersex individuals, who are often viewed as anomalies or subjected to superstitions and myths.
Anthropological studies provide insights into how intersex individuals are perceived and treated in various African communities. Njambi (2009) explores the intersection of

gender, culture, and colonialism, highlighting how indigenous understandings of sex and gender were disrupted by colonial rule. The imposition of Western medical and legal norms has led to the pathologization and marginalization of intersex persons, contrasting with some pre-colonial practices that might have been more accommodating of gender diversity.

2.4.1. Prevalence of Intersex Surgical intervention/(IGM)

Research has shown that Intersex Medical Intervention or (IGM) is a significant issue in several African countries. These non-consensual surgeries are often performed on infants and children to conform their bodies to binary gender norms. Creighton and Minto (2001) highlight that these interventions can lead to severe physical and psychological consequences, including chronic pain, scarring, and trauma. Liao, Wood, and Creighton (2015) discuss the gaps in medical education and the lack of specialized healthcare services for intersex patients.

Many healthcare providers are not equipped to handle ethically or competently cases of intersex, leading to misdiagnosis, stigmatization, and inappropriate treatments.

Intersex individuals in Africa face significant legal challenges due to the absence of specific protections and recognition. Carpenter (2016) notes that most African countries do not have laws that explicitly protect the rights of intersex persons, leaving them vulnerable to discrimination and human rights violations. This legal gap makes it difficult for intersex individuals to obtain accurate legal documents and access essential services.

Human rights organizations and intersex advocacy groups have been working to address these issues. Aniekwu (2006) emphasizes the importance of advocacy in promoting legal reforms and protecting the rights of intersex persons. Efforts by groups such as the Intersex Society of South Africa (ISSA), Intersex Nigeria, Ivy Foundation, Intersex Zambia, Intersex Persons Society of Kenya, SIPD Uganda and international organizations have brought attention to the plight of intersex individuals and pushing for policy changes.

Intersex individuals in Africa often face severe stigmatization and discrimination. Reis (2007) discusses how religious beliefs, misconception, societal and cultural norms, influence attitudes towards intersex persons. This stigmatization leads to social exclusion, violence, and mental health issues, including anxiety, depression, and identity struggles.

2.5. Overview of Intersex Studies in Nigeria

In Nigeria, like in many African countries, cultural beliefs and societal norms stronglyemphasize binary sex and gender roles, often marginalized individuals whose sex characteristics do not conform to these norms. There is a prevalent stigma surrounding

intersex variations, leading to social exclusion, discrimination, and even violence against intersex individuals. Traditional beliefs and superstitions may contribute to misconceptions about intersex traits, further complicating societal acceptance.

Non-consensual medical interventions, including surgeries aimed at altering intersex infants' genitalia to fit binary sex norms, are reported in Nigeria as in the case of kano state government who sponsored 7 infants and adolescents to undergo a nonconsensual corrective' surgical procedure without adequate information or critical understanding of the potential complications(vanguardngr).

These practices are often performed without proper informed consent and can lead to long-term physical and psychological harm.

Intersex individuals in Nigeria do not have specific legal protections or recognition of their unique needs and identities. This lack of legal framework leaves intersex persons vulnerable to discrimination, denial of rights, and challenges in obtaining legal documents that reflect their true identities. Advocacy efforts by local and international organizations are beginning to address these gaps, however a lot more needs to be done.

These efforts aim to raise awareness about intersex issues, advocate for legal reforms, and promote respect for the rights and dignity of intersex individuals in Nigeria. While specific studies on intersex issues in Nigeria are limited, anecdotal evidence and reports from advocacy organizations highlight the urgent need for more research to document intersex variations, needs of intersex people, the impact of medical surgical intervention, and the experiences of intersex individuals across different regions of Nigeria.

Organizations such as Intersex Nigeria and other human rights groups are working to raise awareness about intersex issues, advocate for policy changes, and provide support to intersex individuals and their families. These efforts are crucial in advancing legal protections, improving healthcare access, and fostering societal acceptance of intersex diversity in Nigeria.

Unnecessary Medical Interventions (IGM) is a prevalent and deeply concerning issue in Nigeria. These procedures are often performed on intersex infants and children without giving their parents comprehensive information of the health consequences of these medical interventions. Routine surgeries to alter the genital appearance of intersex individuals are performed without their consent and frequently without fully informing their parents of the potential risks and long-term consequences. The aim of these medical interventions is "normalizing" their genital appearance and hormones to fit societal expectations of male and female bodies. These medical interventions should be delayed if not absolutely necessary, until later in the future when the child has attained adulthood and can give informed consent.

These procedures can include clitoral reductions, vaginoplasties, and gonadectomized, among others. The health complications resulting from unnecessary medical interventions can be severe and lifelong. Intersex individuals who undergo these surgeries often face chronic pain, loss of sexual sensation, scarring, and complications related to urination and reproductive health. Additionally, some intersex individuals require further surgeries throughout their lives to address complications arising from the initial procedures, leading to ongoing medical trauma and suffering as many intersex adults who have undergone "normalising" interventions in childhood have reported dissatisfaction with appearance, scarring, pain, and sexual function that is diminished or absent. Berry and Monro (2022) Intersex traits are often pathologized as medical conditions that need to be corrected, rather than being understood as natural variations of human biology.

This pathologization drives the medical community to recommend, perform surgeries and treatments aimed at "fixing" intersex bodies, rather than supporting the health and well-being of intersex individuals.

The secrecy surrounding intersex and medical interventions further exacerbates the challenges faced by intersex individuals. Medical records are often kept secret, and parents may not be fully informed about their child's condition or the nature of the surgeries being performed. This lack of transparency prevents intersex individuals from accessing their medical histories and understanding the full extent of the interventions they have undergone, hindering their ability to seek appropriate care and support.

There is a limited availability of specialized medical care and counselling for intersex individuals in Nigeria. Healthcare providers who are knowledgeable about intersex variations and capable of providing comprehensive care are scarce. This lack of specialized services means that intersex individuals often do not receive affirming medical attention, services and psychological support they need.

The absence of established guidelines and protocols for the ethical and consensual treatment of intersex individuals is a significant gap in the healthcare system. Without clear guidelines, healthcare providers may rely on outdated and harmful practices that do not prioritize the consent or well-being of intersex patients. Developing and implementing ethical guidelines and protocols is essential to ensure that intersex individuals receive respectful and appropriate care.

3.0. Methodology

This research approaches intersex studies in Nigeria through a comprehensive and systematic method to gather quantitative data, analyze findings, and advocate for policy reforms. This approach is designed to address the gaps in knowledge and prevalence of surgical intervention on intersex, raise awareness, and promote respect for the rights and dignity of intersex individuals. Both quantitative and qualitative data collection methods were employed for the purpose of this research.

To identify hospitals across Nigeria that have encountered intersex births or have performed surgical intervention and other medical interventions. Mapping of at least 50 strategic hospitals across the six geopolitical zones of Nigeria was done and data was collected on the prevalence of instances of surgical intervention on intersex or other medical interventions through structured questionnaires. The selection of hospitals and regions in Nigeria for the study on intersex issues is crucial to ensure comprehensive data collection, representation of diverse healthcare practices, and regional variations.

Selecting hospitals for the study on intersex issues in Nigeria involves careful consideration of various criteria to ensure comprehensive data collection and representation of healthcare practices across different regions. Nigeria is divided into six geopolitical zones:

North-Central, North-East, North-West, South-East, South-South, and South-West. Each zone has unique cultural, religious, and socioeconomic characteristics that influence healthcare delivery and societal perceptions of intersex variations. By selecting hospitals from each zone, the study ensures a comprehensive understanding of the prevalence of intersex birth, prevalence of surgical and medical intervention, and healthcare provider's guidelines on intersex birth in Nigeria.

Hospitals selected for the study are strategically chosen based on their reputation, capacity, and historical records of managing intersex cases. This criterion ensures insights from healthcare providers experienced in intersex care and the availability of relevant data for analysis. For the qualitative aspect of the research, after data were collected through a structured questionnaire, interviews were also conducted for 13 medical doctors to gain insight on the issues of intersex in Nigeria. These 13 respondents are spread across the six Geo-political zone with two respondents in each geopolitical zone except for the southwest were three respondents were interviewed owing to the population concentration in Lagos Nigeria

3.1. Mapping out Hospitals and Medical practitioners.

For a facility to be qualified to be part of this study, the facility must be a hospital ranging from private to federal and state facilities, which have trained professionals that have seen intersex cases. The research approach prioritizes ethical standards by engaging qualified medical professionals. The research looks carefully at two main specialties per facility

which are Paediatrics Specialists and obstetrician and gynaecologists (OBGYN Specialists) but also considered some few professionals who have seen intersex cases including Family Physicians, Paediatrics Surgeons, and General Surgeons.

3.2. Data Collection and Analysis

To gather empirical data on the impact of practices, prevalence of Surgical intervention and societal challenges faced by intersex individuals in Nigeria, we employed 6 medical practitioner for each of the six geopolitical zones in Nigeria who oversee the zone and ensure the recruitment of at least two persons per hospital/state that will participate in the survey.

This is to ensure the inclusion of hospitals from each of Nigeria's six geopolitical zones. We gather data through an administered structured questionnaire to healthcare providers to collect quantitative data on attitudes, practices, and perceptions related to intersex surgical intervention prevalence and rights. To gather qualitative data to gain insight on the issues of intersex interviews were conducted with respondents from six geopolitical zones.

To protect the confidentiality of patient information and respondents we anonymize data during collection, extraction, and analysis, ensuring compliance with ethical guidelines. We analyzed the collected data to derive meaningful insights and inform evidence-based recommendations using statistical methods, to analyze information, such as the prevalence of surgical intervention and frequencies of medical interventions, we ensured data that questions checked each other for accuracy.

The application of Descriptive Statistics to Calculate frequencies percentages and means, to summarize quantitative data related to Surgical intervention and other medical was used. Analysis was done using Statistical Analysis Software (Excel and STATA). Qualitative analysis was done using NVivo which helps organize, code, and analyze text to identify patterns and themes.

4.0. Findings and Discussion

4.1. Demography of the respondent

The research conducted in 50 facilities in the six Geo-political Zones of Nigeria was responded to by medical professionals. The demography of the respondent is seen below.

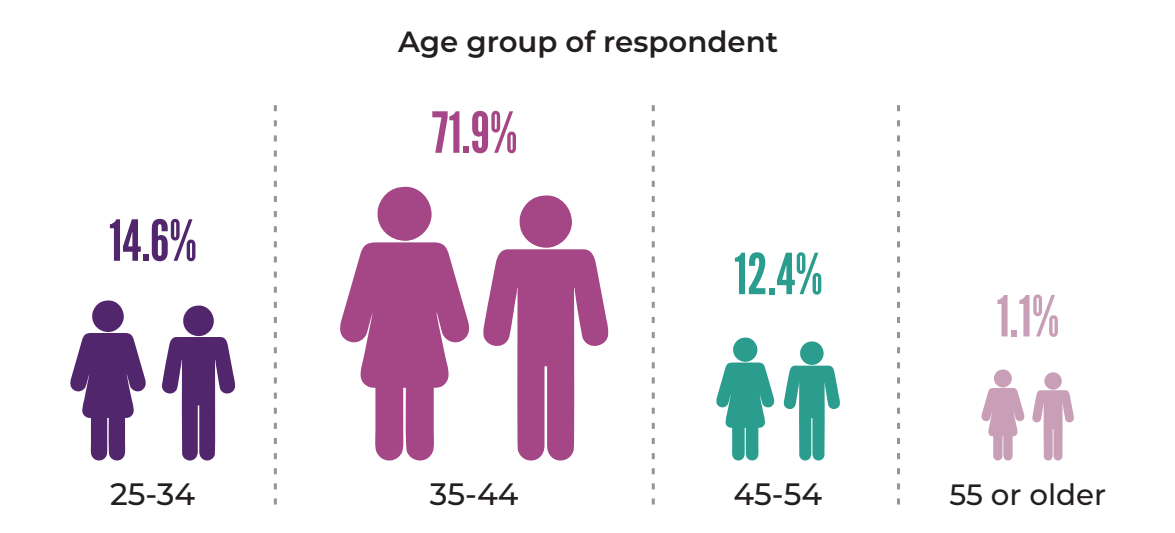


Figure 1. Shows the Age range of the respondents.

The age range "35- 44 years" is overwhelmingly the most common, comprising about 71.9% of the respondents. This shows a strong focus or representation of medical practitioners in their early age of practice. The age ranges "25-34" and "45-54" represent 14.6% and 12.4% of the respondent, respectively. Only a small percentage of "55 or older" which is about 1% of the respondents participated in the survey.

4.2. Regional Representation and Specialties

The whole 6 Geopolitical zones in the country were represented with the mean representation of 2 respondents per state.
This means that, on average, each state has at least 2 respondents in the survey.

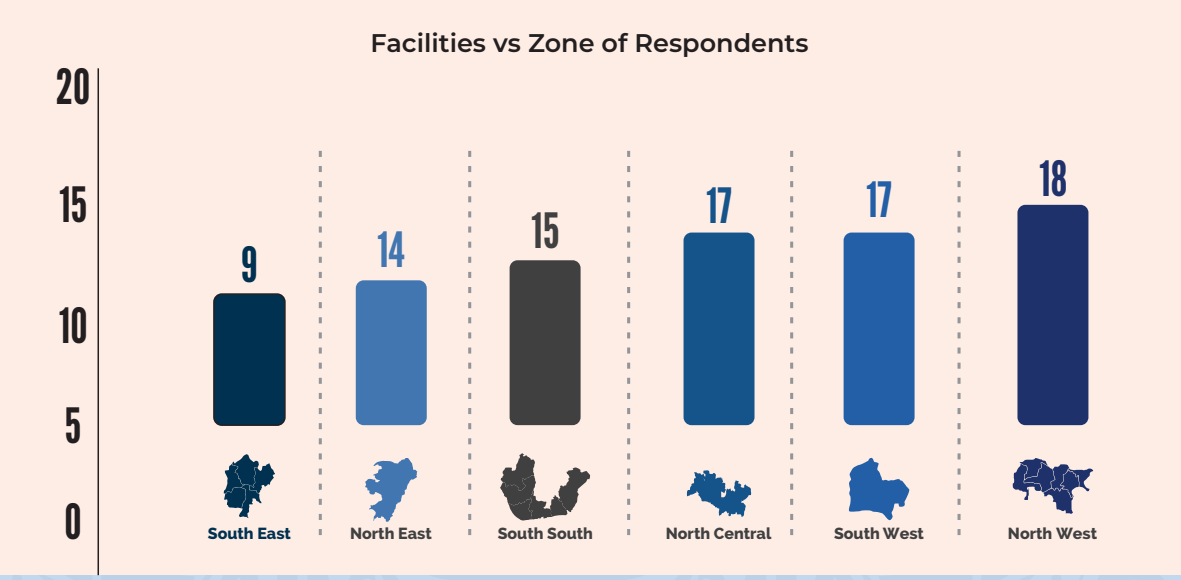


Figure 2. Shows the number of facilities in each zone of the respondents.

From Figure 2 above it is evident that the North-West region has the highest number of respondents which is 18, while the South-East has the lowest 9.

The North-Central and South-West regions both have 17 respondents each, while the South-South and North-East regions have 15 and 14 respondents respectively.
Overall, there is participation from all zones in the country with at least one respondent per state.

The levels of health facilities where the respondents practice are all tertiary and public facilities which indicates that the respondents of this research are all professionals from public settings in Nigeria.

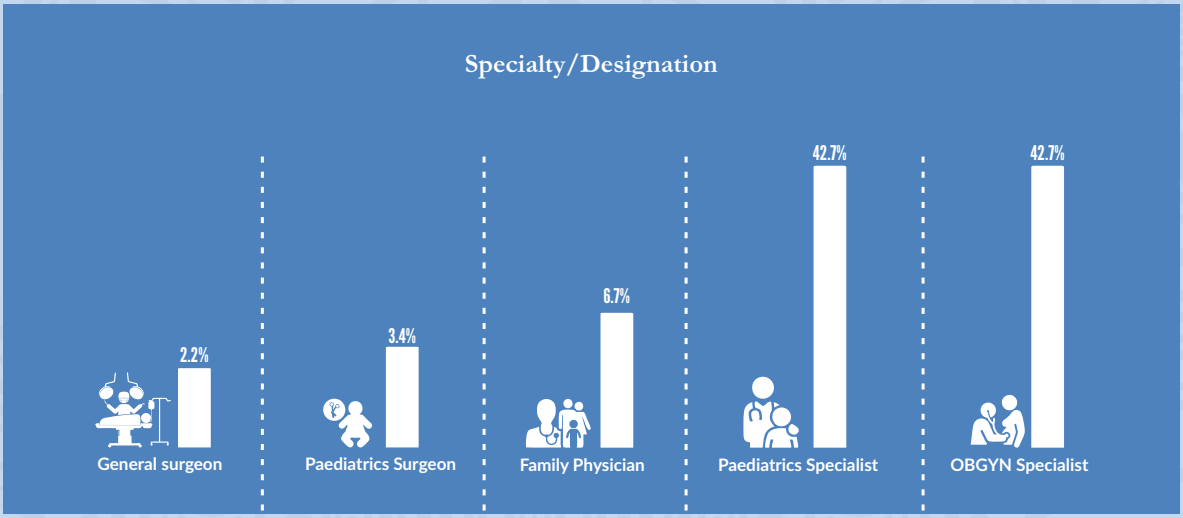


Figure 3: Shows the designation and specialty of all respondents.

Most Represented Specialties are Paediatrics Specialists (42.7%) and OBGYN Specialists (42.7%). Family Physicians represent about (6.7%) of the sample representation Paediatrics Surgeons represent about (3.4%) and General Surgeons represent (2.2%) representation of the sample respondents.

The dataset representation is heavily skewed towards Paediatrics Specialists and OBGYN Specialists, indicating a primary focus on these specialties.

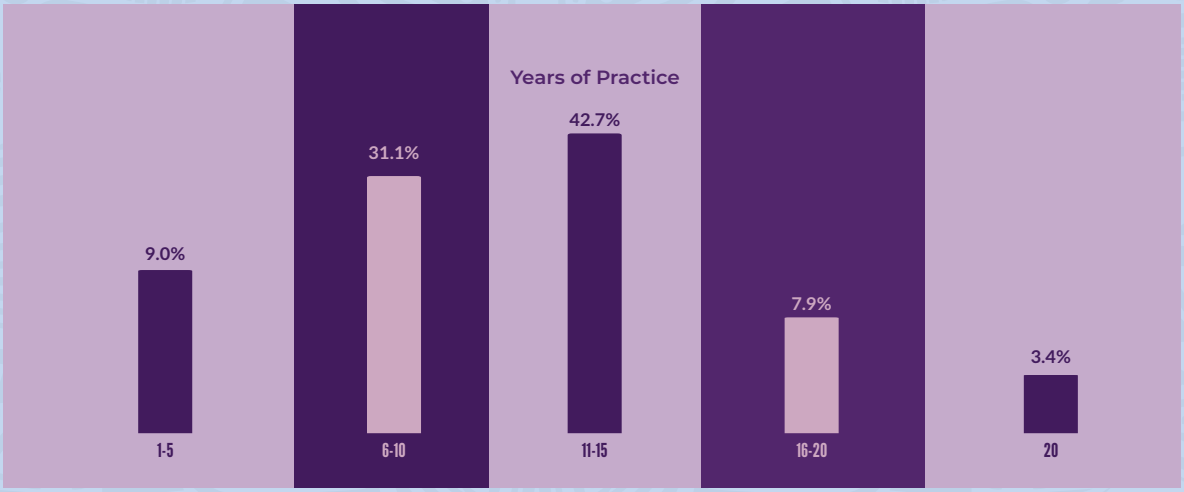


Figure 4: Shows Years of practice of the respondents

The 11-15 years range is the most common, with nearly half of the individuals (42.7%) respondents. The 6-10 years range also has a significant representation of about (37.1%), indicating that most of the respondents have mid-level to relatively high experience in their field. Few respondents fall within the 16-20 years range (7.9%) and (9.0%) respondents are in the 1-5 years range. A little of about 3.4% of the respondents have more than 20 years of experience.

4.3. Awareness and Knowledge of Intersex

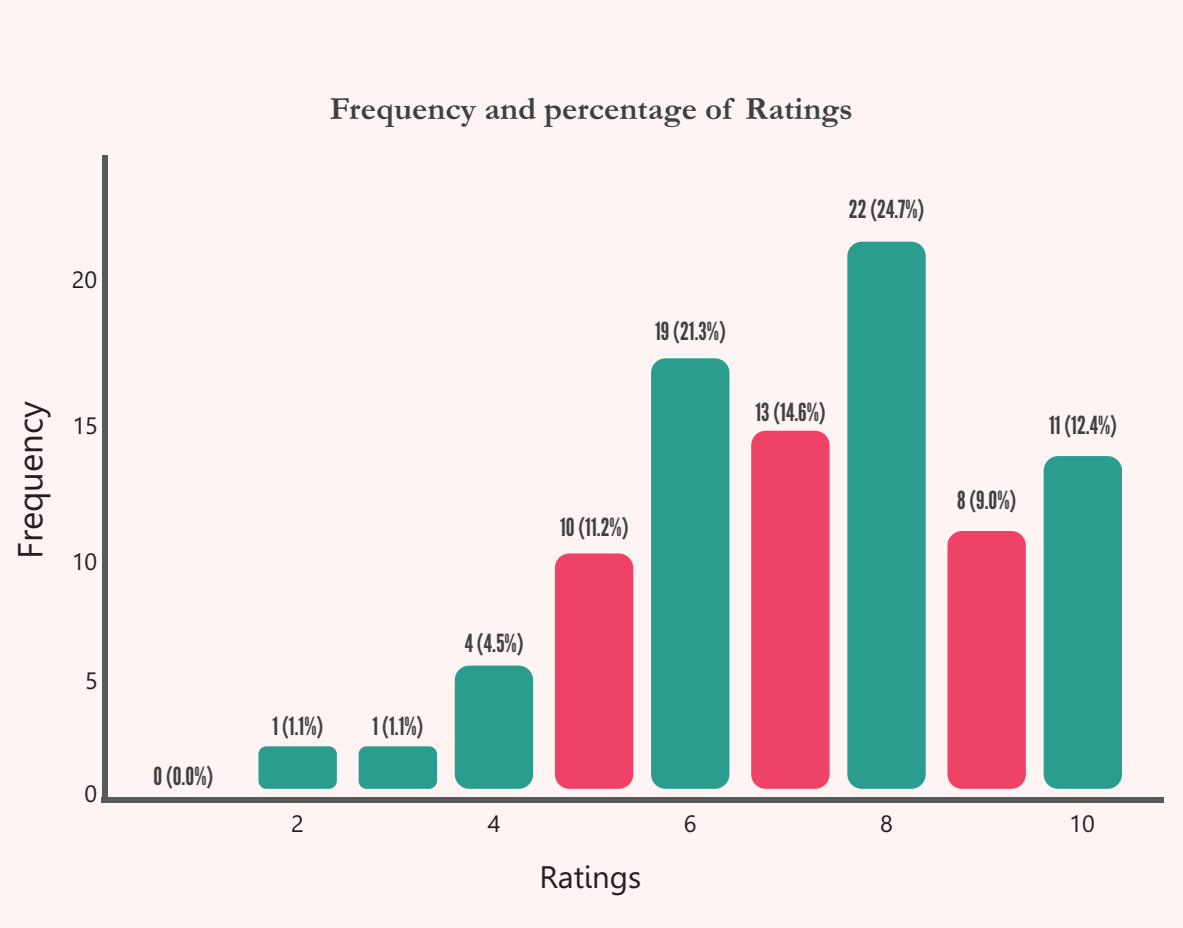


Figure 5: Shows the scale of knowledge of respondents on intersex

Figure 5 shows that about 54 (61%) of respondents rate themselves on a scale of 7 to 10,meaning they consider themselves very knowledgeable about intersex.

33 respondents, which is about 37% rate themselves between 4 and 6, suggesting a significant number of respondents with moderate knowledge.

And a few comprising 2% rate themself between 2 and 3 considering themselves to have shallow knowledge of intersex.

4.4. Prevalence of Intersex Births

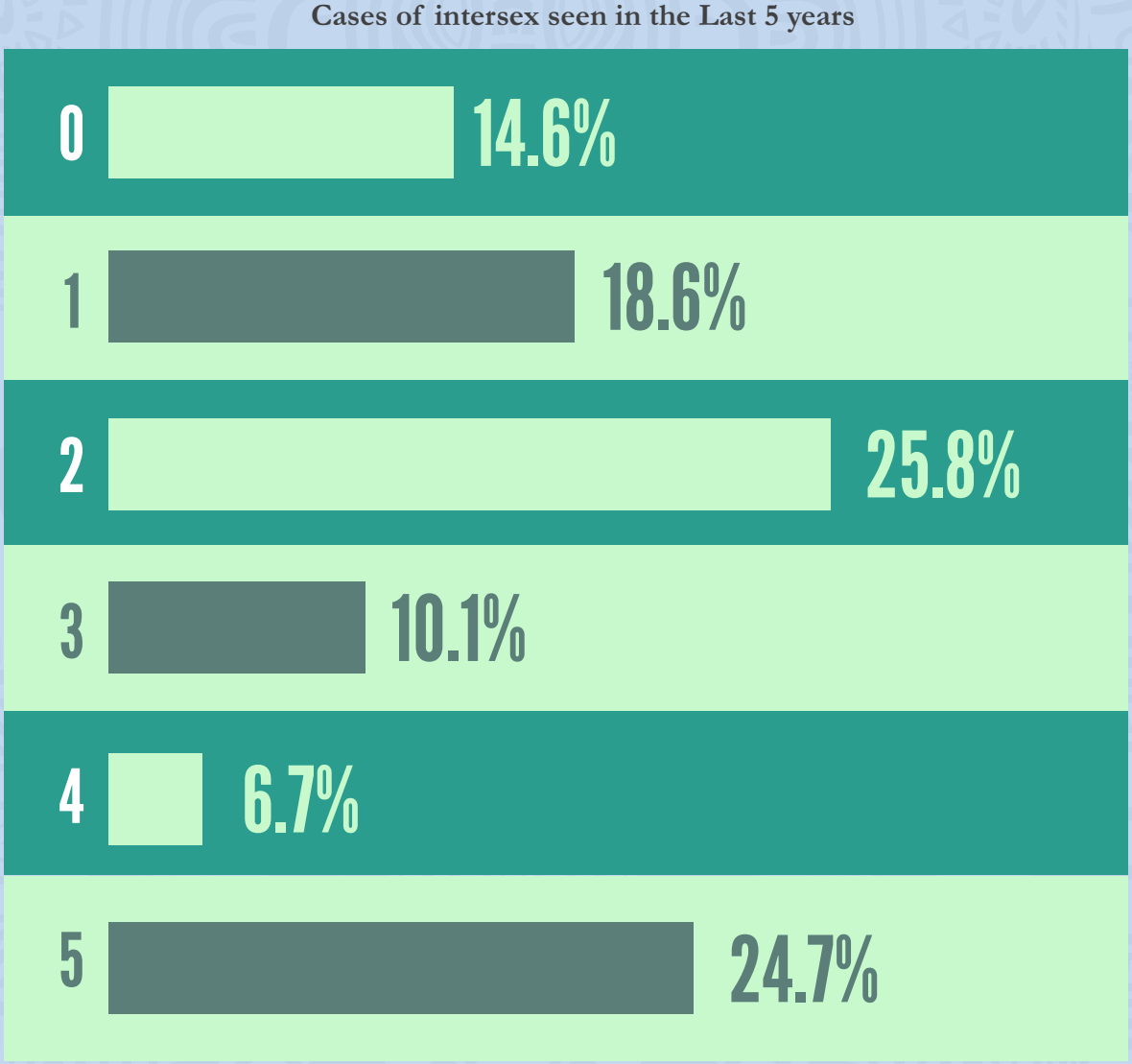


Figure 6 shows the cases of intersex seen in the past 5 years by respondents

About 15% of respondents have not seen any intersex cases in the past 5 years of their professional career, and 18% of respondents have seen just one intersex case. The most common observation is those who have seen 2 cases, and this is about 26% of respondents.

10% of respondents have seen 3 intersex cases and 7 % of respondents have seen 4 intersex cases in the last 5 years. Notably, one quarter which is 25% of respondents has seen 5 or more intersex cases in the past 5 years of their career.

This goes to show that the occurrence of intersex birth is very significant as about 85% of the respondents have seen at least 1 case of intersex in the last five years.

4.5. Clinical Evaluation and Treatment on Intersex Persons

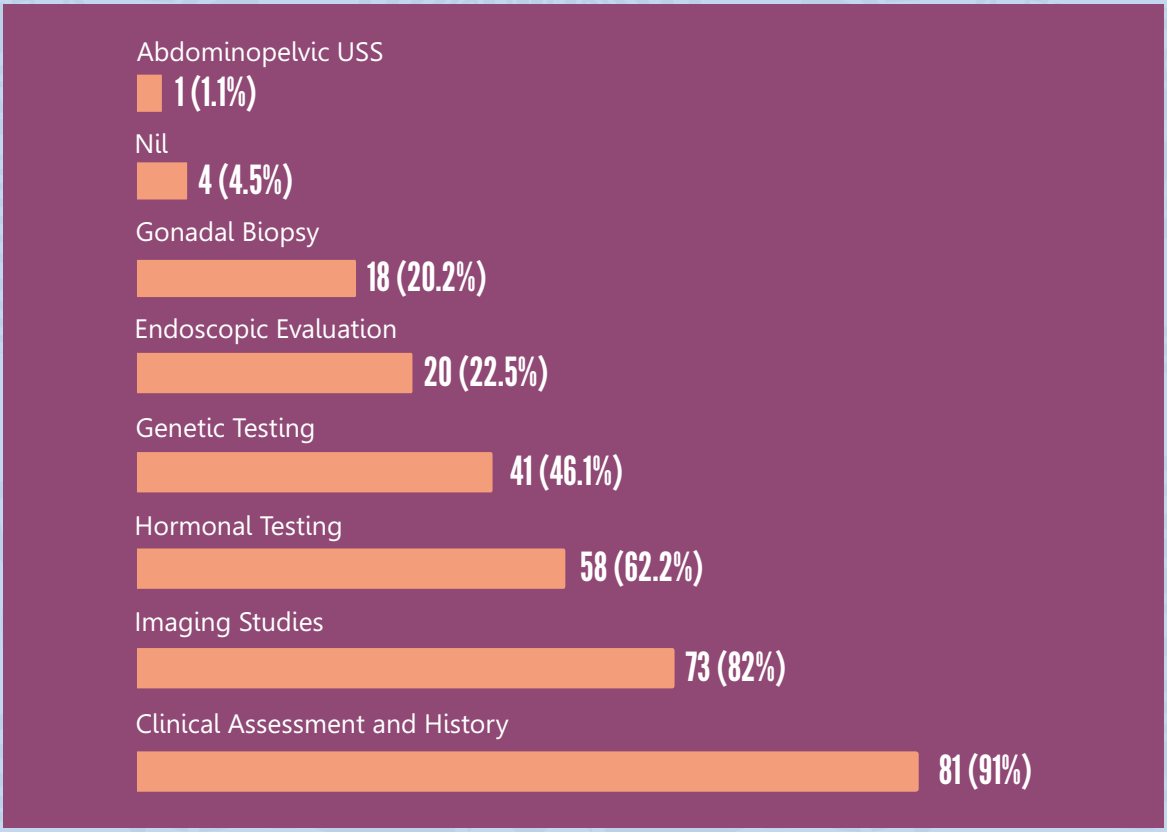


Figure 7 shows the diagnostic criteria and methods used to identify intersex.

Since respondents are allowed to select more than one option as it applies in their facility, Clinical Assessment and History (91%) and Imaging Studies (82%) are the frequently used diagnostic methods, indicating a high reliance on these clinical practices for identifying intersex.

Hormonal Testing which is about (65.2%) is also a common diagnostic method, reflecting its importance in evaluating intersex. It can also be seen that Genetic Testing which nearly half of the respondents (46%) is another diagnostic method used. Endoscopic Evaluation (22.5%) and Gonadal Biopsy (20.2%) are less commonly used but are still notable methods for some specific cases.

A small percentage of respondents (1.1%) use methods like checking for Barr bodies or abdominopelvic ultrasound.

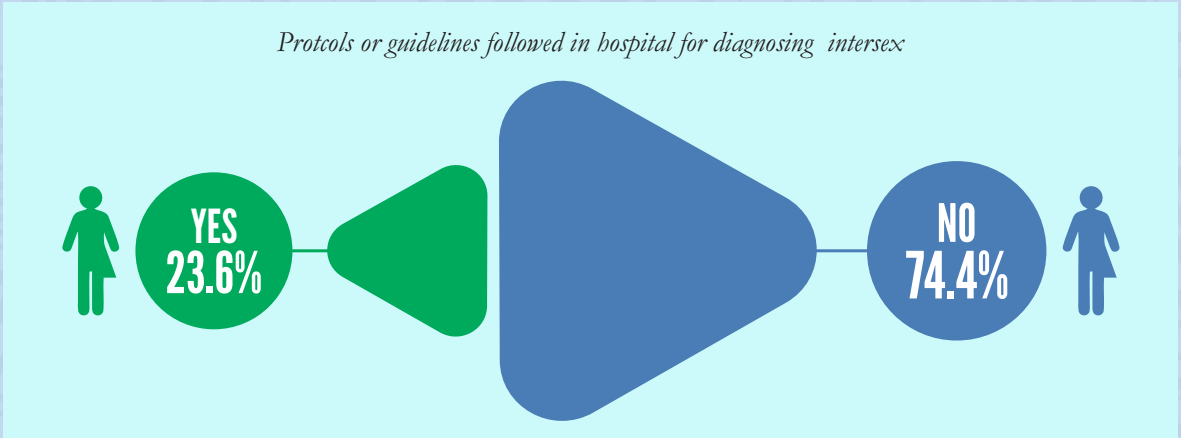


Figure 8: shows if there are specific protocols or guidelines followed for diagnosing intersex.

Twenty-one respondents (23.6%) revealed that there are specific protocols or guidelines followed in their hospitals for diagnosing intersex. while sixty-eight respondents (76.4%) indicated that there are no specific protocols or guidelines followed in their hospitals for diagnosing intersex.

This shows that the majority of public facilities do not have specific protocols or guidelines in place for diagnosing intersex. An in-depth analysis shows that most of the 76.4% of the states in Nigeria don't have specific protocols or guidelines in place in their hospitals for diagnosing intersex variations.

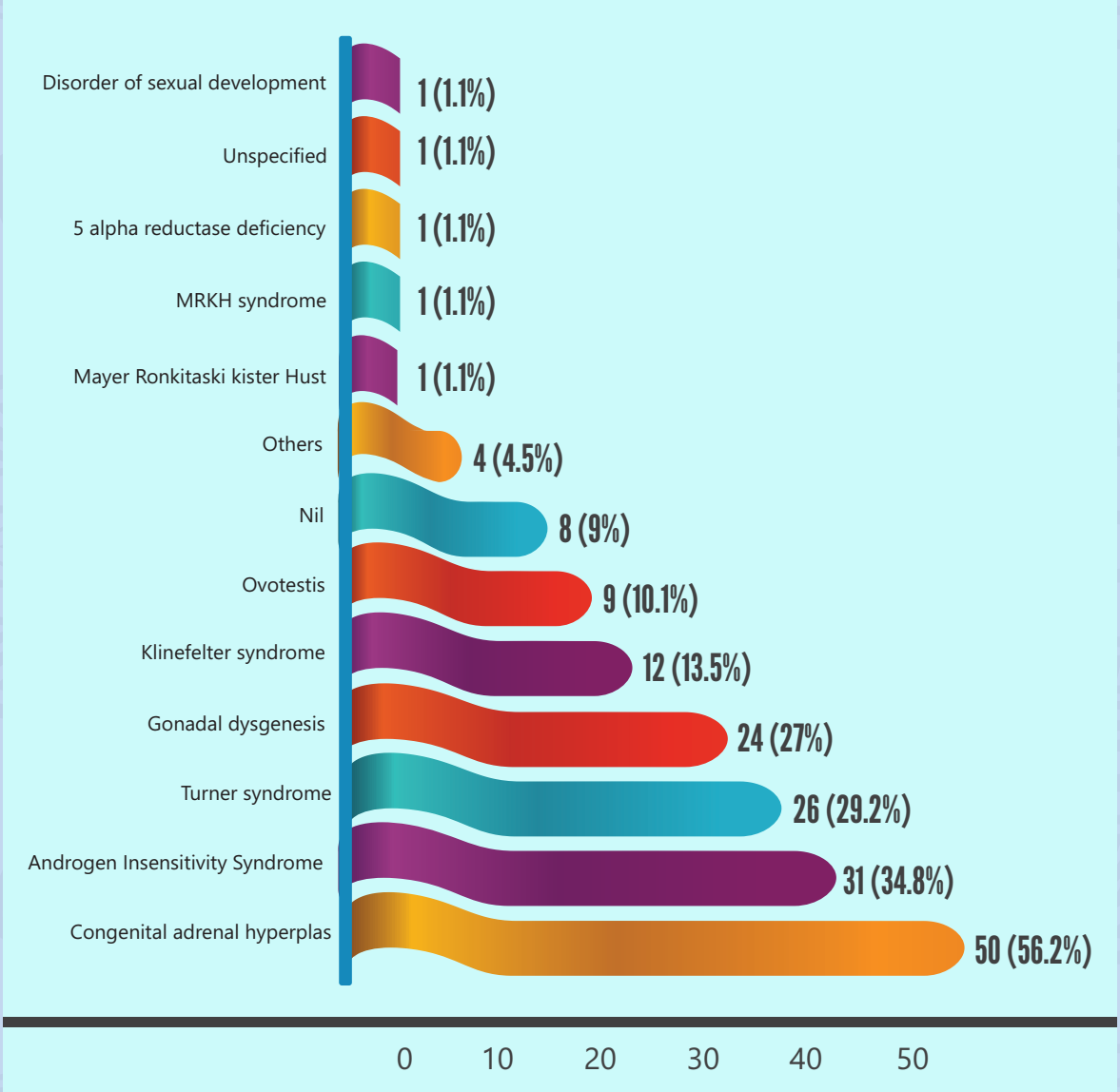


Figure 9: shows the diagnosis of cases seen by respondents.

Congenital Adrenal Hyperplasia (CAH) is the most frequently reported diagnosis, present in about 56.2% of the cases. Androgen Insensitivity Syndrome (AIS) is also a common diagnosis, seen in 34.8% of cases. Turner Syndrome and Gonadal Dysgenesis are equally prevalent, each found to be approximately 26% and 24% respectively. Klinefelter Syndrome represents 12% of cases diagnosed, and Ovotestis is noted in 10.1% of cases. Mayer-Rokitansky-Küster-Hauser (MRKH) syndrome, 5 Alpha Reductase Deficiency, and Disorder of Sexual Development (DSD) are relatively rare, with each accounting for less than 1 or 2% of the cases and Others represent a small portion at 4.5%.

Figure 10: shows the most recent diagnosis seen by the respondent

Congenital Adrenal Hyperplasia (CAH) is the most common and recent diagnosis that most have done and this accounts for 47.6% of the cases. Androgen Insensitivity Syndrome (AIS) is the second most common recent diagnosis, which accounts for 14.3% of cases. Turner Syndrome is reported to occur in 10.7% of cases.

Congenital Adrenal Hyperplasia (CAH) is the most prevalent recent diagnosis among the reported diagnoses, followed by Androgen Insensitivity Syndrome (AIS) and Turner Syndrome.

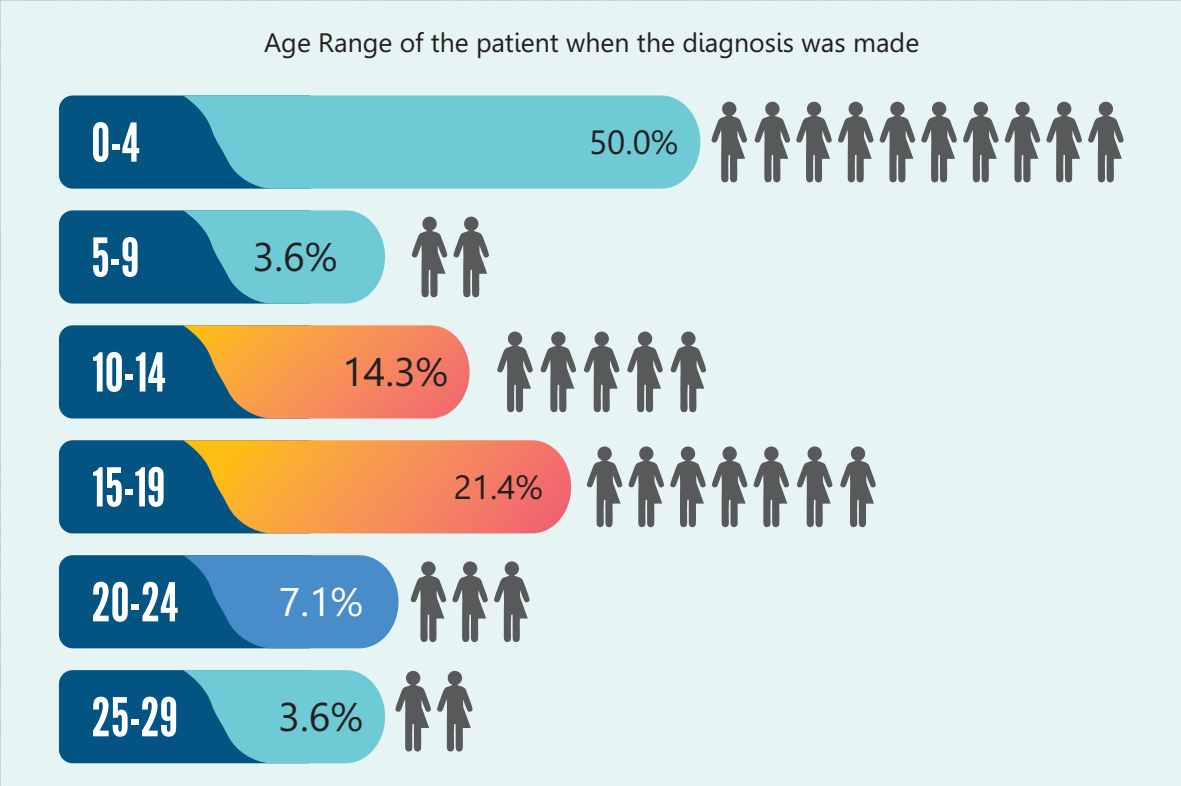


Figure 11: shows the age of the patient when the recent diagnosis was made

Most of the cases diagnosed were found in patients between 0-4 years accounting for 50% of the diagnosis of recent cases. This shows that half of the diagnoses made are children. The 15-19 years age group accounts for 21.1% of diagnoses also showing a significant number of diagnoses occurring during adolescence.

The age group 10-14 years is reported in approximately 14.3% of diagnoses. Approximately 68% of cases diagnosed are between the age of 1-14 which is quite a huge percentage of the population who are not in the position to make decisions about their bodies.

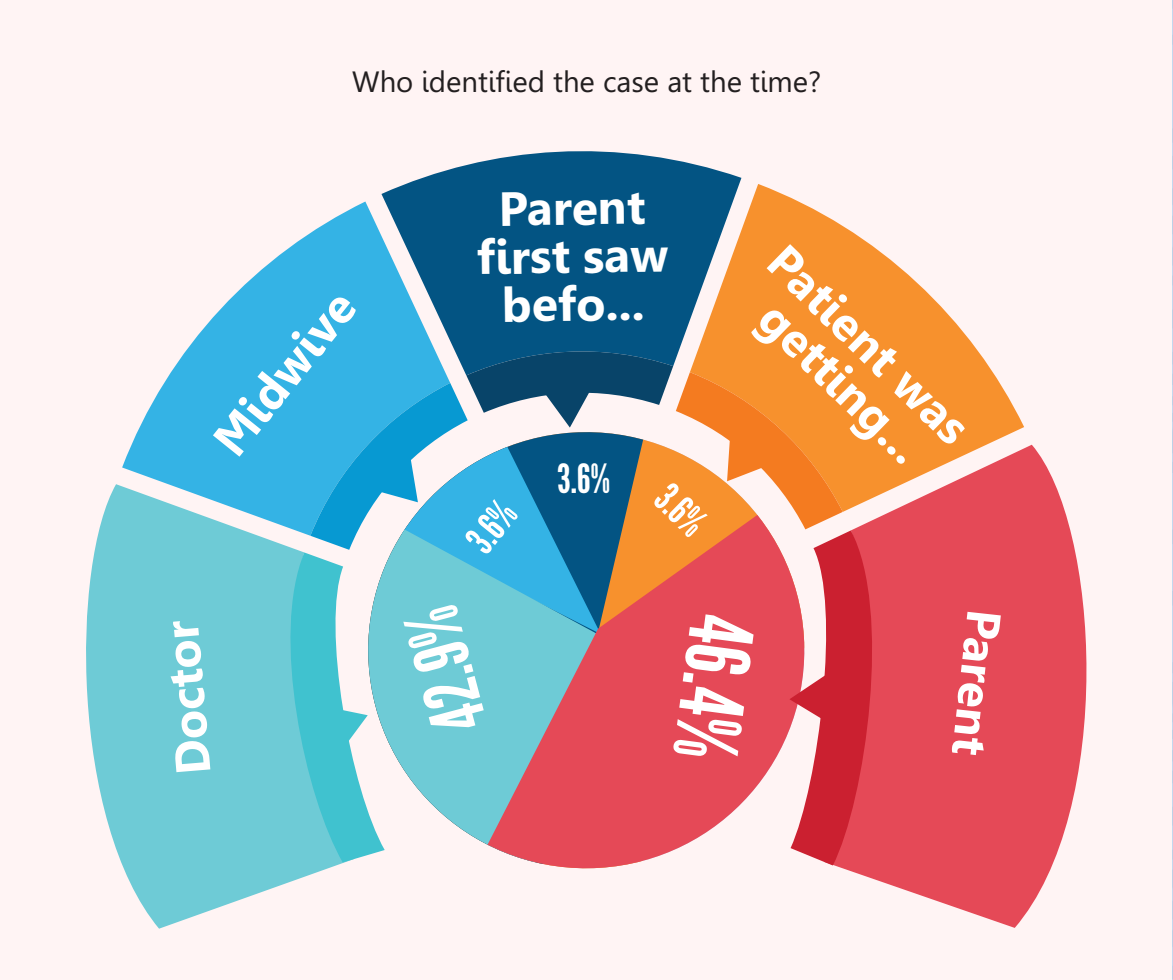


Figure 12: Shows responses of who identified the case.

From Figure 12 above 46.4% of the intersex cases were first identified by parents, highlighting the role of familial observation in early identification while doctors make up about 43%. Midwives and patients themselves played a minimal role in initial identification with each approximately 4%.

"There is an account of a Patient who was about to get married and had clitoromegaly which was noticed at birth. But didn't present it to the hospital until when she was supposed to bemarried. Her Parents first saw them before the diagnosis was made by the Doctors."

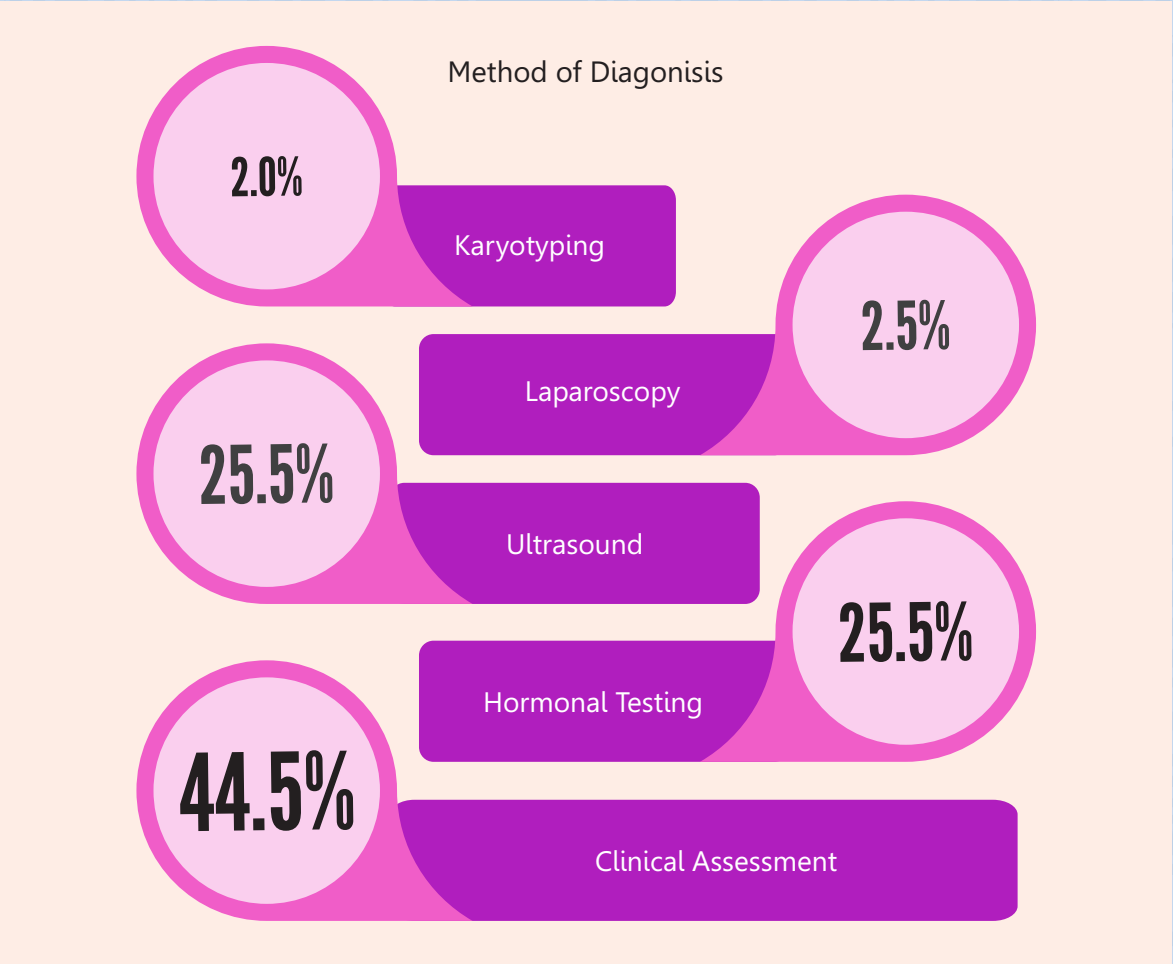


Figure 13: shows the method used in the diagnosis of the most recently seen case.

Hormonal Testing, Ultrasound and Clinical Assessment and History are the most common methods of diagnosis in recent cases with Clinical assessment being more prevalent.
Laparoscopy and Karyotyping are also used in a few cases. Other Methods of diagnosis used less frequently include Gonadal Biopsy, MRI, and Laboratory Investigation.

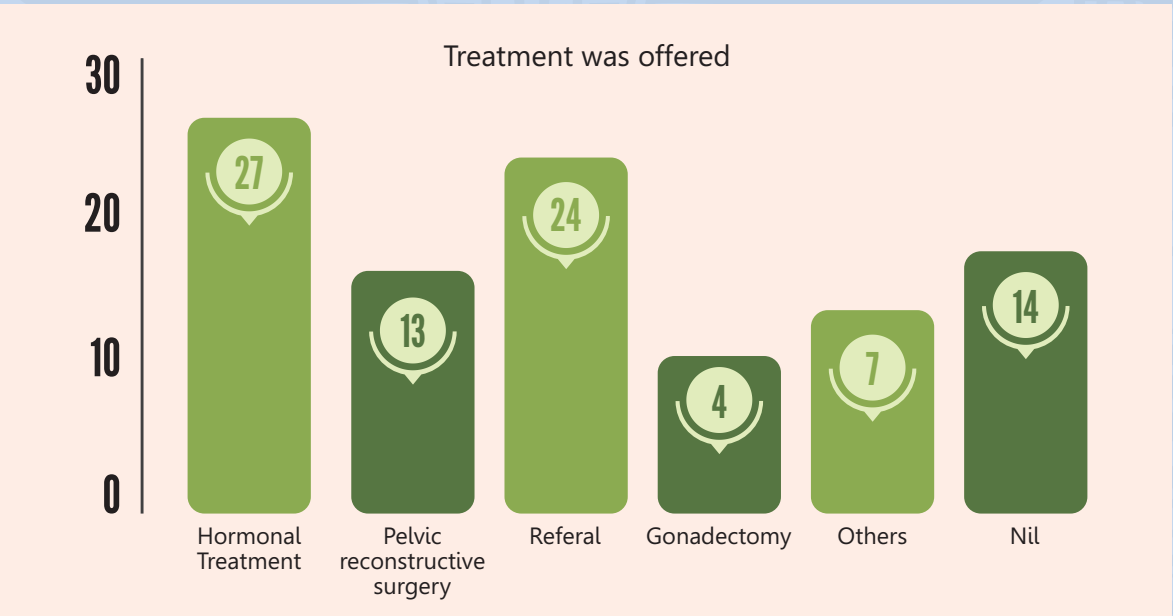


Figure 14: Shows the Medical interventions offered by professionals on the most recently seen case

The medical interventions offered to patients diagnosed shows that Hormonal therapy was offered to 27 (30.3%) of the cases, making it the most common treatment.

Pelvic Reconstructive Surgery was offered for 13 cases which make up 14.6% of the cases. About 24 (27%) cases were referred by the physician. Gonadectomy was offered to 4 (4.5%) of the cases. There are other medical interventions not mentioned which make up 7.9% of the cases and some professionals haven't treated any cases.

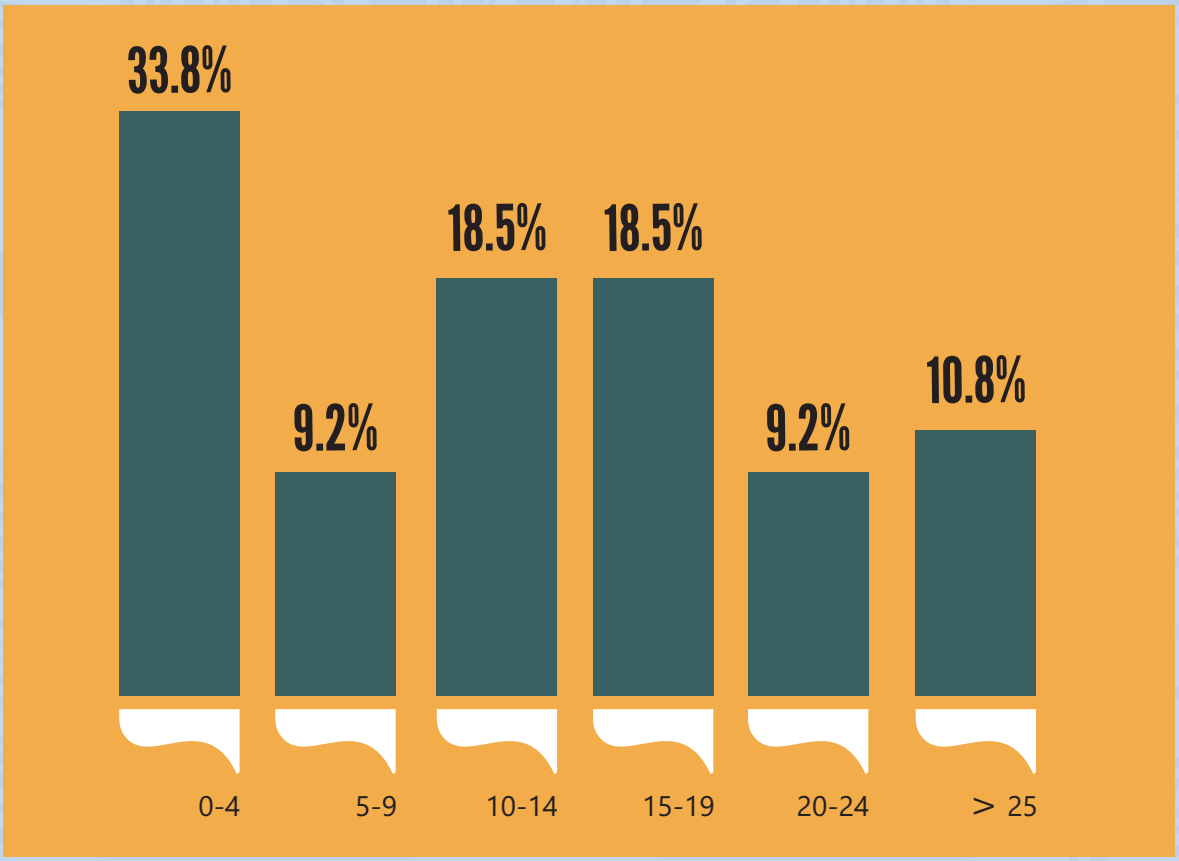


Figure 15: Shows the age of the intersex when medical interventions were done.

Most medical interventions done in recent times occurred in the 0-4 years age group, followed by the 10-14 years and 15-19 years groups. Treatments were less common in the older age groups. Even though we have over 10% treated at over 25 years of age.

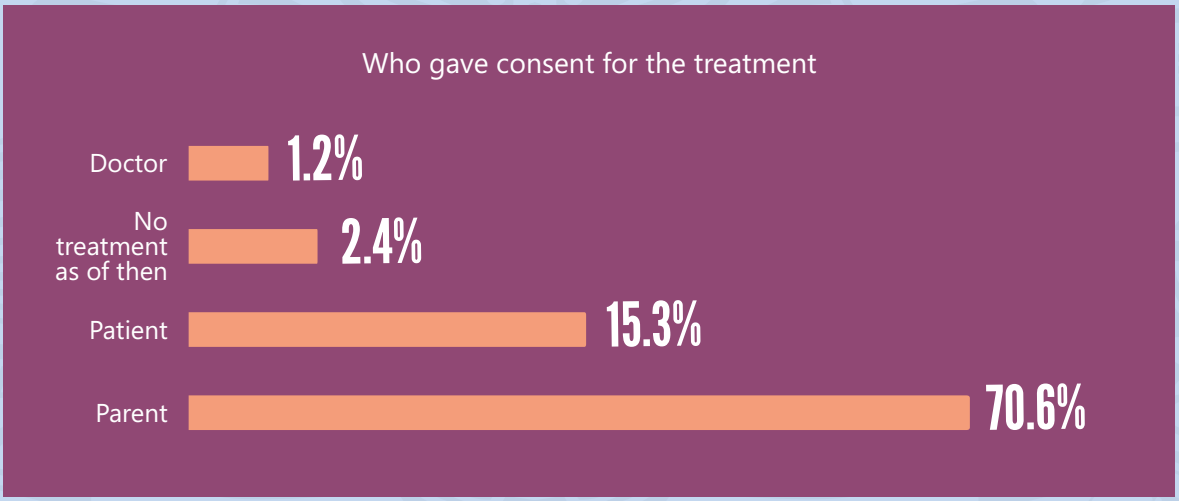


Figure 16: Shows who gave consent for the medical interventions .

Consents were mostly given by parents of the children which is approximately 71% of the cases, followed by the patients themselves for patients over the age of 18 years. In a few cases, no treatment had been administered at the time of reporting.

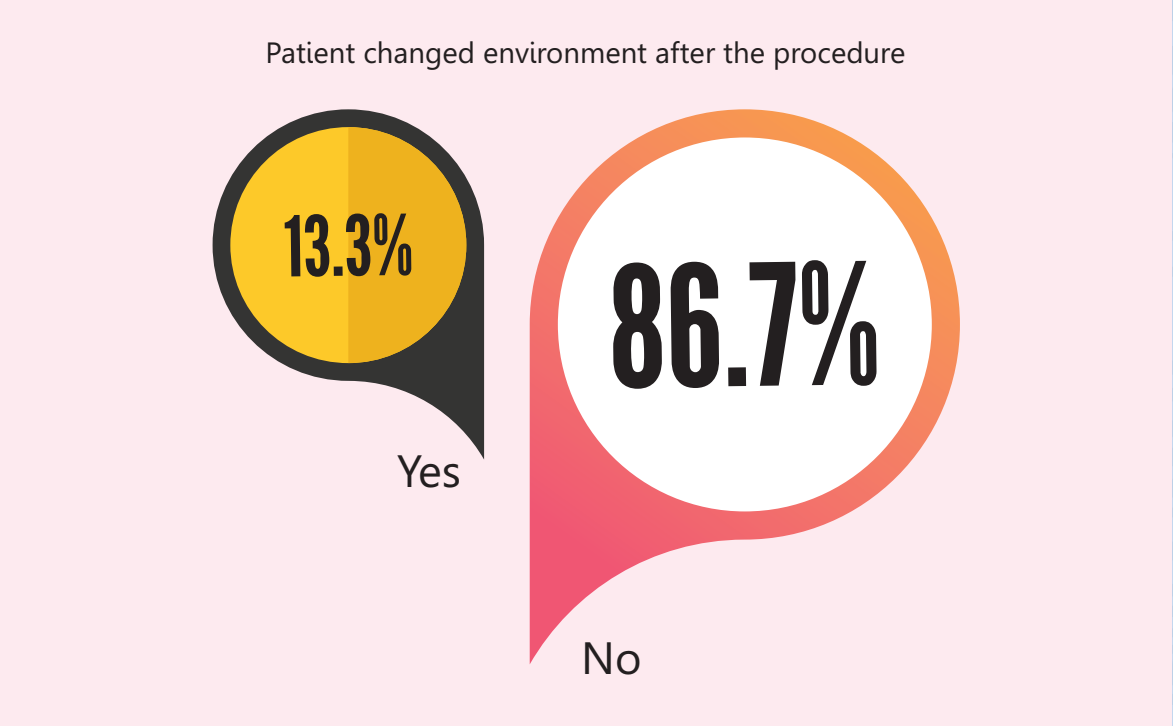


Figure 17: Shows if the patient changed environment after the procedure.

The chart above shows that most patients did not change their environment after the procedure. While a few, about 13% changed their environment after the procedure.

4.6. Attitudes and Practices of Healthcare Providers

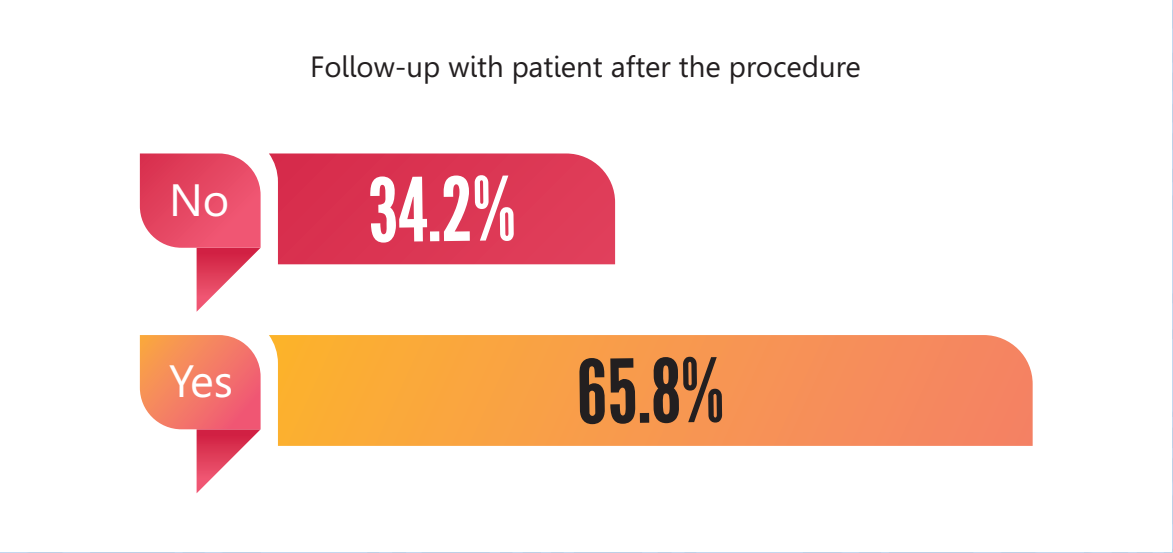


Figure 18: Shows if there was a follow-up after the procedure.

About 66% of respondents provided follow-up care in most cases, with about one-thirds of patients not receiving follow-up after their procedure.

4.7. Societal and Cultural Influences on Intersex Medical Interventions

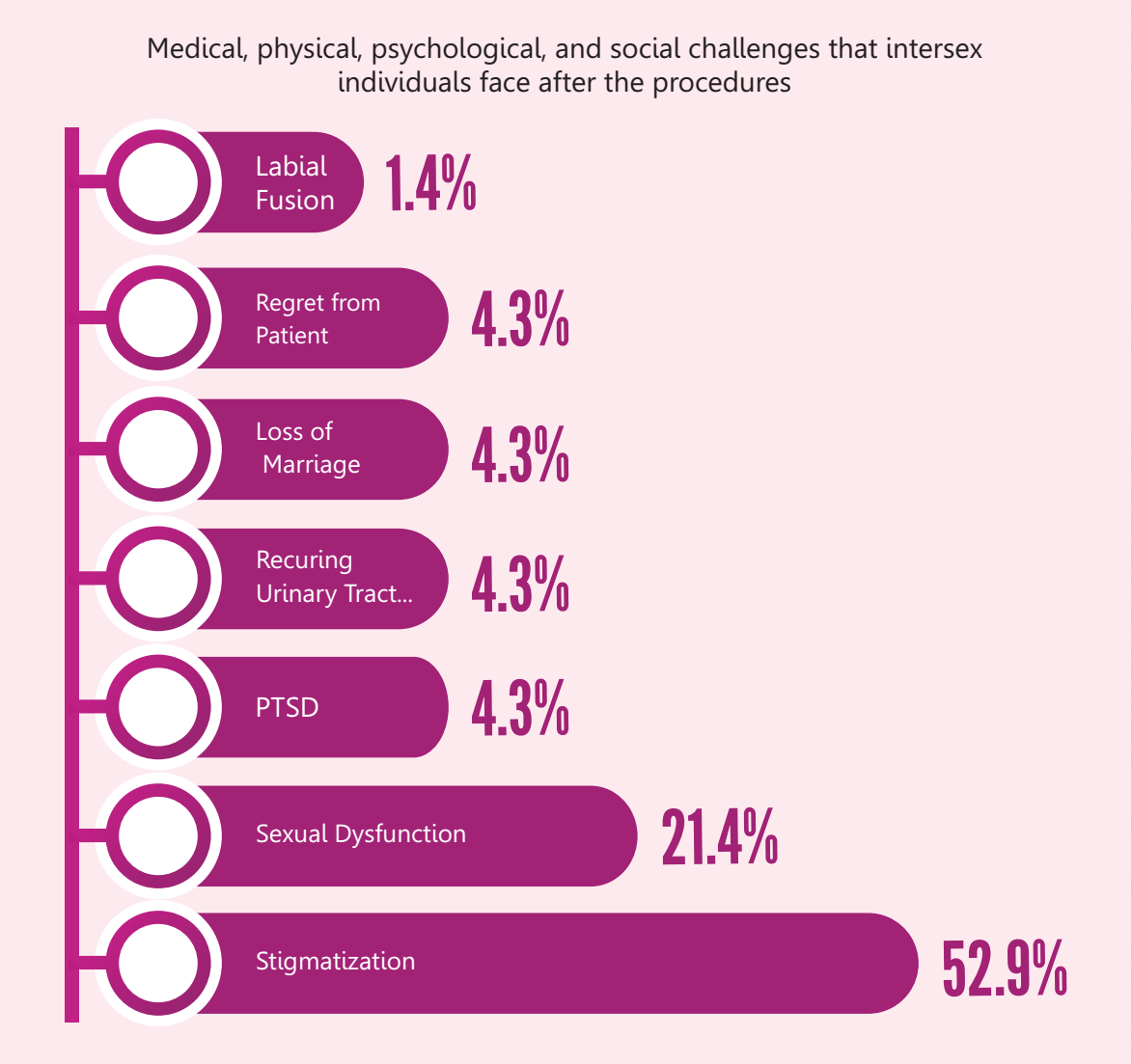


Figure 19: shows the challenges faced by intersex after treatment.

The most common challenge faced by intersex individuals after procedures is stigmatization, affecting 53% of the cases. Sexual dysfunction is also a significant issue, reported by 21% of respondents. Other notable challenges include loss of marriage, Post-traumatic stress disorder (PTSD), and recurring urinary tract infections, each affecting a smaller percentage of individuals.

A significant portion of the cases reported no challenges or identified no challenges due to various reasons.

4.8. Discussion of quantitative findings

The majority of respondents rate themselves as having a high level of knowledge about intersex, with about two-thirds scoring themselves between 7 and 10 on a scale of 1-10. This reveals that most healthcare professionals consider themselves well-informed, which is crucial for the effective diagnosis and management of intersex. However, over one-third of respondents rate their knowledge between 4 and 6, indicating that a significant

proportion may only have a moderate understanding, which could affect the quality of care provided.

The presence of a few who rate their knowledge very low highlights the need for ongoing education and training to ensure all healthcare professionals are adequately equipped to handle intersex.

Intersex births are relatively common, with most respondents having seen at least one case in the past five years. This prevalence underscores the importance of having knowledgeable healthcare providers and standardized protocols in place. The most common diagnostic methods are Clinical Assessment and History, and Imaging Studies, also other diagnostic methods include Hormonal Testing and Genetic Testing which are being less applied.

A significant gap exists in the availability of specific protocols or guidelines for diagnosing intersex, with many respondents indicating their hospitals lack such standards. This gap can lead to inconsistencies in care and potentially result in misdiagnosis or inappropriate interventions. The need for developing and implementing standardized guidelines is evident to ensure uniform and accurate diagnosis across all healthcare facilities.

The majority of diagnoses occur in early childhood, with half made in the 0-4 years age group. This early diagnosis is crucial for timely intervention but also raises ethical considerations regarding consent and the potential impact of early medical interventions on individuals who are too young to make informed decisions about their bodies.

Stigmatization is the most common challenge faced by intersex individuals after medical interventions, affecting over half of cases. Sexual dysfunction, loss of marriage, PTSD, and recurring urinary tract infections are also reported as significant challenges. These findings show the profound social and psychological impact of being intersex, particularly after medical interventions.

The fact that over a quarter of patients were not satisfied with the sex assigned to them shows the importance of involving intersex individuals in decisions about their treatment and the need for more supportive and informed care practices.

4.9. Discussion of interview findings

Healthcare professionals treating intersex individuals face significant challenges in balancing patient autonomy, ethical decision-making, religious and cultural dilemmas. The clash between what is medically advisable and what is culturally acceptable leads to complex ethical issues, particularly around autonomy, paternalism, and justice. The management of intersex individuals in Nigeria presents significant challenges that require healthcare providers to balance ethical considerations, cultural sensitivities, and medical imperatives.

While some hospitals have had success, many face significant barriers, including late presentation, cultural resistance, lack of advanced diagnostic tools, and the cost of care.

The most common challenge faced by intersex individuals after procedures is stigmatization, affecting 53% of the cases. Sexual dysfunction is also a significant issue, reported by 21% of respondents. Other notable challenges include loss of marriage, Post-traumatic stress disorder (PTSD), and recurring urinary tract infections, each affecting a smaller percentage of individuals.

A significant portion of the cases reported no challenges or identified no challenges due to various reasons.

> *Intersex is not a common presentation in the clinic and, probably because it has been underreported.*

4.9.1. Challenges and ethical considerations involved in treating intersex individuals.

One key challenge and ethical considerations involved in treating intersex individuals is autonomy which is the right of patients to make decisions about their medical care. Intersex patients, particularly children, often have their sex assigned by caregivers.

This early assignment can compromise their autonomy as they grow older. Tension may arise when patient preferences clash with medical recommendations, requiring healthcare professionals to navigate the delicate balance between respect for autonomy and medical advice.

> *There are no clearly laid down ethical principles, for their management, within our hospital*
>
> *I don't know of any policy by the hospital.*

In balancing autonomy, beneficence, non-maleficence, and justice, healthcare practitioners face challenges when cultural beliefs influence medical decisions. Often, compelled to guide families towards medically sound choices, despite the risks of overriding personal belief. This shows the difficulties medical practitioners face in ensuring ethical care when societal and cultural norms conflict with medical recommendations.

The management of intersex individuals in Nigeria presents significant challenges that require healthcare providers to balance ethical considerations, cultural sensitivities, and medical imperatives. Addressing these issues involves a careful approach, with an emphasis on education, support, and careful navigation of the ethical landscape.

> *The child, most times, is not in the know of the decisions. So the parents are really the ones who make such decisions.*

> *We make sure that whatever you want to do is what they agree with you to do for them.*

4.9.2. Experiences with intersex in terms of diagnosis and treatment approach in Nigeria

Clinical presentation and imaging techniques like ultrasound scans are frequently used as the first-line diagnostic tools. In some cases, hormonal assays and imaging are available, especially in more resource-equipped hospitals. The unavailability of advanced diagnostic techniques, such as karyotyping and genetic testing was a major concern, and this is due to the high cost and lack of infrastructure in many Nigerian hospitals. Another significant challenge is that many intersex children are born at home or present late to the hospital. This leads to delayed diagnoses.

There is a clear disparity between the diagnostic and treatment modalities available in Nigerian hospitals and the needs of intersex patients.

This mismatch impacts the quality of care, as the system lacks infrastructure and trained personnel in many areas. The lack of public awareness and understanding of intersex conditions poses a significant barrier to early diagnosis. This is compounded by cultural and religious factors, which further delay appropriate healthcare interventions.

> *Usually, one of the challenges is that some parents cannot afford to even do the investigation. Then it's a long term management. It's almost like something that goes on for life.*

4.9.3. Impact of cultural and societal beliefs in the management and treatment of intersex in Nigeria

One of the beliefs that impact management of intersex individuals is the preference of a particular sex over another.

When there is still a strong cultural preference for male children, this bias often leads parents to choose a male identity for intersex children, even when medical evidence may suggest otherwise. This leads to late presentations and complications in managing intersex individuals.

Religious beliefs also strongly influence how intersex are perceived and managed. Some people believe that these conditions are "God's will" and may resist medical intervention, feeling it is an attempt to change God's plan. In many parts of Nigeria, parents are often too shy or ashamed to discuss issues related to their child's genitalia, delaying diagnosis and treatment. The lack of early recognition of intersex conditions due to societal stigma leads to late diagnosis and complicates management.

> *"One of the ways to go about treatment of intersex people is to try and educate people if they understand. If they don't understand and if they don't want the treatment, allow them to go"*

Intersex conditions are sometimes considered a taboo, and people may avoid seeking help out of fear of societal judgment. Some children with ambiguous genitalia may be hidden or even abandoned due to the belief that such conditions are linked to witchcraft or spiritual attacks.

There is significant social stigma around intersex individuals. This often leads to the isolation of intersex people, contributing to mental health issues like depression, stigmatization, and rejection by society. Many doctors Emphasized the importance of educating communities on the existence of intersex conditions to reduce the stigma and improve timely medical interventions.

4.10. Effects of Surgical Intervention on Intersex Persons

I. Physical Impacts: Many intersex individuals report difficulties with sexual function following surgical interventions, which can include loss of sensation, pain during intercourse, and difficulties with sexual arousal. There are also reports of Recurrent Infections, Surgeries, particularly those involving the urinary tract or genital reconstruction, can lead to recurrent urinary tract infections. Hormonal treatments offered to patients which are often used to align physical characteristics with assigned gender, can cause various side effects and require ongoing management and the risks associated with surgical procedures include scarring, chronic pain, and complications requiring additional surgeries.

II. Mental Impacts: Stigmatization and societal rejection can lead to a poor self-image and low self-esteem and this is one of the experiences of intersex individuals. Many individuals experience significant psychological stress due to identity confusion, dissatisfaction with their bodies, and the trauma of medical interventions. The invasive nature of early surgeries and the secrecy often surrounding their condition can lead to PTSD. High rates of depression and anxiety are prevalent among intersex individuals, often related to the stress of living with their condition and societal rejection.

III. Social Impacts: Intersex individuals frequently face stigmatization and discrimination in various aspects of life, including school, work, and within their communities. In many cultures, the inability to meet traditional gender and reproductive expectations can lead to challenges in forming marital relationships. The Fear of rejection and bullying often leads to social withdrawal and isolation. Families of intersex individuals often experience distress and societal pressure, which can affect family dynamics and support systems.

5.0. Conclusion

These research findings reveal several critical insights into the diagnosis, unnecessary medical interventions , and ongoing challenges faced by intersex. Early diagnosis, predominantly by parents, and a reliance on multiple diagnostic methods such as clinical assessments, hormonal testing, and ultrasounds are common trends. Treatment typically involves hormonal therapy and surgery, with parental consent being the norm but this r aises concerns about the potential impact of early medical interventions on individuals who are too young to make informed decisions about their bodies instead these decisions are made on their behalf.

Intersex individuals face significant challenges, including stigmatization, sexual dysfunction, and psychological issues. Satisfaction with assigned sex and treatment varies, with some individuals expressing dissatisfaction due to inadequate care. The lack of consistent follow-up care further aggravates these issues, highlighting the need for improved healthcare practices and societal support.

The lack of standardized protocols, combined with the varied levels of knowledge among healthcare providers, can lead to inconsistent care and outcomes for intersex persons. This then highlights the need for the development of protocols and guidelines distributed across facilities to be used to treat intersex.

The findings also emphasize the need for improved education and training for healthcare professionals, greater awareness and sensitization towards the unique needs of intersex individuals. Addressing these gaps will help ensure that intersex persons receive consistent, compassionate, and informed care, reducing the negative impact of medical interventions and societal stigma on their lives.

5.1. Recommendations

i. **Development and Implementation of Standardized Protocols:** There is an urgent need for the establishment of standardized protocols and guidelines for diagnosing and managing intersex cases across all healthcare facilities. These protocols should be evidence-based, human rights sensitive, and designed to ensure consistent and accurate care for intersex individuals. Collaboration with intersex persons, intersex organizations, healthcare providers, Federal Ministry of Health, and local stakeholders can help in developing these guidelines, ensuring they are relevant and applicable within the Nigerian context.

ii. **Enhanced Training and Education for Healthcare Professionals:** The varying levels of knowledge among healthcare providers indicate a need for comprehensive education and training programs focused on intersex. These programs should cover the medical, ethical, human rights and psychosocial aspects of intersex care, enabling

healthcare professionals to provide informed, empathetic, and competent care. Continuous professional development should also be encouraged to keep healthcare workers updated on the latest developments in intersex healthcare.

iii. **Multidisciplinary Care Approach:** Establishing specialized teams, including pediatricians, surgeons, psychologists, and counselors, will ensure comprehensive management of intersex individuals. This approach should include accessible counseling services to guide both patients and families through decision-making, ensuring a more ethical and balanced care process.

iv. **Increased Awareness and Reduce Stigma:** Efforts should be made to raise awareness about intersex both within the healthcare system and the broader society. Public education campaigns can help reduce stigma and discrimination against intersex individuals, promoting an inclusive and supportive environment. Within healthcare settings, sensitization programs should be implemented to address the biases and misconceptions that may affect the quality of care provided to intersex patients.

v. **Involvement of Intersex Individuals in Decision-Making:** It is crucial to involve intersex individuals, where possible, in decisions regarding their healthcare. For younger patients, efforts should be made to consider their future autonomy, and decisions should be made in their best long-term interests. This patient-centred approach should respect the rights and dignity of intersex persons and ensure that any medical interventions are carried out with their informed consent or that of their guardians.

vi should ensure that follow-up care is provided, including psychological support services to help intersex individuals cope with the social and emotional impacts of their condition and treatment. This support should be continuous and tailored to meet the unique needs of intersex individuals throughout their lives.

vii. **Research and Data Collection:** There is a need for more extensive research and data collection on intersex in Nigeria. This will help to better understand the prevalence, outcomes, and challenges faced by intersex individuals. Such data is essential for informed policy-making and for improving healthcare practices related to intersex care. Partnerships with academic institutions, healthcare organizations, and intersex advocacy groups can help drive research efforts and ensure that they are inclusive and representative.

viii. **Policy Advocacy and Legal Protections:** Policies should be developed to protect intersex individuals from harmful practices and discrimination and to ensure they have access to appropriate healthcare. Legal frameworks should also be established to safeguard the rights of intersex children, particularly concerning consent to medical treatments.

References

African commission's resolution on the rights of intersex persons in africa (2023), what it means for state and non-state actors in africa: *A guide to resolution 552 on the promotion and protection of the rights of intersex persons in africa. pg 1.*
https://www.chr.up.ac.za/images/researchunits/sogie/documents/Resolution_552_booklet_final-2.pdf

Report of the Special Rapporteur on torture and other cruel, inhuman or degrading treatment or punishment, Juan E. Méndez "77"
https://www.ohchr.org/sites/default/files/Documents/HRBodies/HR Council/RegularSession/Session22/A.HRC.22.53_English.pdf

Amadiume, Ifi (1987). Male Daughters, Female Husbands: Gender and Sex in an African Society. Zed Books.

Aniekwu, Nkoli. (2006). The Convention on the Elimination of All Forms of Discrimination Against Women and the Status of Implementation on the Right to Health Care in Nigeria.

Anne Tamar-Mattis, Arlene Baratz, Katharine Baratz Dalke & Katrina Karkazis , Psychology & Sexuality (2013): Emotionally and cognitively informed consent for clinical care for differences of sex development, Psychology & Sexuality, DOI:10.1080/19419899.2013.831215

Berry AW, Monro S. Ageing in obscurity: a critical literature review regarding older intersex people. Sex Reprod Health Matters. 2022 Dec;30(1):2136027. doi: 10.1080/26410397.2022.2136027. PMID: 36369786; PMCID: PMC9704076.

Carpenter M. The human rights of intersex people: addressing harmful practices and rhetoric of change. Reprod Health Matters. 2016 May;24(47):74-84. doi: 10.1016/j.rhm.2016.06.003.
Epub 2016 Jul 12. PMID: 27578341.

Creighton, S. M., Michala, L., Mushtaq, I., & Yaron, M. (2013). Childhood surgery for ambiguous genitalia: glimpses of practice changes or more of the same? Psychology & Sexuality, 5(1), 34–43. https://doi.org/10.1080/19419899.2013.831214

Creighton SM, Minto CL, Steele SJ. Objective cosmetic and anatomical outcomes at adolescence of feminising surgery for ambiguous genitalia done in childhood. Lancet. 2001Jul 14;358(9276):124-5. doi: 10.1016/S0140-6736(01)05343-0. PMID: 11463417.

Davis R, Campbell R, Hildon Z, Hobbs L, Michie S. Theories of behaviour and behaviour change across the social and behavioural sciences: a scoping review. Health Psychol Rev. 2015;9(3):323-44. doi: 10.1080/17437199.2014.941722. Epub 2014 Aug 8. PMID: 25104107; PMCID: PMC4566873.

Dreger, A. D. (1998). "Ambiguous Sex"—Or Ambivalent Medicine?: Ethical Issues in the Treatment of Intersexuality. Hastings Center Report, 28(3), 24-35 https://doi.org/10.2307/3528648

Esteban C, Ortiz-Rodz DI, Muñiz-Pérez YI, Ramírez-Vega L, Jiménez-Ricaurte C, Mattei-Torres E, Finkel-Aguilar V. Quality of Life and Psychosocial Well-Being among Intersex-Identifying Individuals in Puerto Rico: An Exploratory Study. Int J Environ Res Public Health. 2023 Feb 7;20(4):2899. doi: 10.3390/ijerph20042899. PMID: 36833596; PMCID: PMC9957316.

Fausto-Sterling, A. (2000). Sexing the Body: Gender Politics and the Construction of Sexuality. New York: Basic Books

Garland, Jameson & Slokenberga, Santa. (2018). Protecting the Rights of Children with Intersex Conditions from Nonconsensual Gender-Conforming Medical Interventions: The View from Europe. Medical Law Review. 27. 10.1093/medlaw/fwy039.

Holmes M 2011, The Intersex Enchiridion: Naming and Knowledge Somatechnics, September 2011, vo. 1, No. 2 : pp. 388-411

Karkazis, Katrina. (2008). Fixing Sex: Intersex, Medical Authority, and Lived Experience. 10.1515/9780822389217.

Liao LM, Wood D, Creighton SM. Parental choice on normalising cosmetic genital surgery. BMJ. 2015 Sep 28;351:h5124. doi: 10.1136/bmj.h5124. PMID: 26416098.

Njambi, Wairimu. (2009). 'ONE VAGINA TO GO'. Australian Feminist Studies - AUST FEMINIST STUD. 24. 167-180. 10.1080/08164640902852381.

Oyěwùmí, O. (1997). *The Invention of Women: Making an African Sense of Western Gender Discourses* (NED-New edition). University of Minnesota Press. http://www.jstor.org/stable/10.5749/j.ctttt0vh

Reis, H. T. (2007). Steps toward the ripening of relationship science. Personal Relationships, 14(1), 1–23. https://doi.org/10.1111/j.1475-6811.2006.00139.x

https://www.vanguardngr.com/2021/04/kano-govt-sponsors-surgery-of-7-patients-with-inters ex-problem/

List of Charts and Tables

Figure 1. *Shows the Age range of the respondents.*

Figure 2. *Shows the number of facilities in each zone of the respondents.*

Figure 3: *Shows the designation and specialty of all respondents.*

Figure 4: *Shows Years of practice of the respondents*

Figure 5: *Shows the scale of knowledge of respondents on intersex*

Figure 6 *Shows the cases of intersex seen in the past 5 years by respondents.*

Figure 7 *Shows the diagnostic criteria and methods used to identify intersex.*

Figure 8: *Shows if specific protocols or guidelines are followed for diagnosing intersex.*

Figure 9: *Shows the diagnosis of cases seen by respondents.*

Figure 10: *Shows the most recent diagnosis seen by the respondent*

Figure 11: *Shows the Age of the patient when the recent diagnosis was made.*

Figure 12: *Shows responses of who identified the case.*

Figure 13: *Shows the method used in the diagnosis of the most recently seen case.*

Figure 14: *Shows the Treatment offered by professionals on the most recently seen case.*

Figure 15: *Shows the age of the intersex when treatment was done.*

Figure 16: *Shows who gave consent for the treatment.*

Figure 17: *Shows if the patient changed environment after the procedure.*

Figure 18: *Shows if there was a follow-up after treatment.*

Figure 19: *shows the challenges faced by intersex after treatment.*

Table 1: *Definition of terms*

Table 2: *Number of respondents per state and Zone*

Table 3: *Name of facility and number of respondent*

Appendices

Table 1: Definition of terms

Terms	Definition
Intersex Genital Mutilation (IGM)	A Medical or surgical intervention performed on intersex individuals, to alter their genitalia to conform to binary sex norms. These procedures can have lifelong physical and psychological consequences.
Androgen Insensitivity Syndrome (AIS)	A condition in which a person who is genetically male (XY chromosomes) is resistant to male hormones (androgens). As a result, they may have female physical traits despite having male genetic makeup.
Congenital Adrenal Hyperplasia (CAH)	A group of genetic disorders affecting the adrenal glands, leading to an overproduction of androgens. It can result in ambiguous genitalia in newborn females and early puberty in both sexes.
Klinefelter Syndrome	A genetic condition affecting males, where an extra X chromosome (XXY instead of XY) causes physical, developmental, and reproductive issues, including reduced testosterone production and infertility.
Turner Syndrome	A chromosomal disorder in females where part or all of one of the X chromosomes is missing. It can lead to short stature, ovarian dysfunction, and other health issues.
Gonadal Dysgenesis	A condition in which the gonads (ovaries or testes) do not develop properly, leading to impaired sexual development and often resulting in infertility.
Ovotestis	A rare condition where an individual has both ovarian and testicular tissue in the same or opposite gonads, leading to ambiguous genitalia and mixed sexual characteristics.
Mayer-Rokitansky-Küster-Hauser (MRKH) Syndrome	A disorder affecting females, characterized by the underdevelopment or absence of the uterus and upper part of the vagina, despite having normal external genitalia and ovarian function.
5 Alpha Reductase Deficiency	A rare genetic condition in which a male is born with external genitalia that may appear female or ambiguous due to a deficiency in the enzyme 5-alpha reductase, which is crucial for male sexual development.
Disorder of Sexual Development (DSD)	A group of congenital conditions where the development of chromosomal, gonadal, or anatomical sex is atypical, leading to ambiguous genitalia or differences in sexual development.
Clitoromegaly	An abnormal enlargement of the clitoris, which can occur as a result of congenital conditions like CAH or due to hormonal imbalances.
Labia Fusion	A condition where the labia minora or majora fuse together, often due to congenital factors or hormonal imbalances, leading to complications with urination and sexual function.
Clinical Assessment and History	A fundamental part of the diagnostic process, involving a thorough physical examination and review of the patient's medical history to identify signs and symptoms of a condition.
Endoscopic Evaluation	A minimally invasive procedure using an endoscope (a flexible tube with a light and camera) to examine internal organs and structures, such as the gastrointestinal tract or reproductive organs.
Gonadal Biopsy	A surgical procedure where a small sample of tissue is taken from the gonads (ovaries or testes) for examination under a microscope to diagnose or assess reproductive health.
Abdominopelvic Ultrasound	A non-invasive imaging technique that uses sound waves to create images of the organs within the abdomen and pelvis, often used to assess the reproductive organs.
Imaging Studies	Refers to various techniques (e.g., ultrasound, MRI, CT scans) used to create images of the body's internal structures to diagnose or monitor medical conditions.
Genetic Testing	Laboratory analysis of DNA to identify genetic disorders or conditions, often used to diagnose syndromes like AIS, CAH, or Turner Syndrome.

Hormonal Testing	Blood tests that measure hormone levels to diagnose or monitor conditions related to endocrine function, including intersex conditions.
Laparoscopy	A minimally invasive surgical procedure where a camera is inserted into the abdomen through small incisions to examine the organs and perform surgeries.
Karyotyping	A laboratory procedure that examines the size, shape, and number of chromosomes in cells, used to diagnose genetic conditions like Klinefelter Syndrome or Turner Syndrome.
Magnetic Resonance Imaging (MRI)	A non-invasive imaging technique that uses magnetic fields and radio waves to create detailed images of the body's organs and tissues, often used for diagnosing complex conditions.
Laboratory Investigation	General term for various tests conducted in a lab (e.g., blood tests, urinalysis) to diagnose or monitor health conditions.
Hormonal Treatment	The use of hormone therapy to treat conditions related to hormone imbalances or deficiencies, such as hormone replacement therapy for CAH or AIS.
Pelvic Reconstructive Surgery	Surgical procedures aimed at correcting or reconstructing the pelvic organs, often used in cases of congenital abnormalities or after trauma.
Gonadectomy	The surgical removal of the gonads (testes or ovaries), sometimes performed in cases of intersex conditions to reduce cancer risk or as part of gender confirmation surgery.
Sexual Dysfunction	Difficulty or inability to experience normal sexual function, which can result from physical, psychological, or medical conditions, including those related to intersex conditions.
Post-Traumatic Stress Disorder (PTSD)	A mental health condition triggered by a traumatic event, which can affect individuals who have undergone invasive medical procedures or experienced stigmatization.

Table 2: Number of respondents per state and Zone

Region	State	Number of Respondent
North-East	Bauchi	3
	Gombe	2
	Adamawa	2
	Yobe	2
	Borno	2
	Taraba	3
Total North East		14
North-West	Kebbi	2
	Sokoto	2
	Jigawa	2
	Kano	6
	Kaduna	2
	Katsina	2
	Zamfara	2
Total North-West		18
North-Central	Plateau	2
	Niger	2
	Kogi	3
	FCT	4
	Kwara	2
	Benue	2
	Nasarawa	2
Total North-Central		17
South-East	Imo	2
	Ebonyi	1
	Anambra	3
	Enugu	1
	Abia	2
Total South-East		9
South-West	Oyo	2
	Osun	2
	Ondo	2
	Ekiti	2
	Ogun	3
	Lagos	6
Total South-West		17

Table 2: Number of respondents per state and Zone

Region	State	Number of Respondent
South-South	Bayelsa	2
	Cross River	4
	Delta	2
	Rivers	3
	Akwa Ibom	2
	Edo	2
Total South-South		15

Table 3: Name of facility and number of respondents

Name of Hospital	Number of respondent
Ahmadu Bello University Teaching Hospital Zaria	2
Alex Ekwueme federal teaching hospital Abakaliki	1
ALEX EKWUEME FEDERAL TEACHINGS HOSPITAL ABAKALIKI (AEFTHA)	1
Aminu kano Teaching Hospital	4
ATBUTH, Bauchi	2
Benue State University Teaching Hospital Makurdi	2
Dalhatu Araf Specialist Hospital	2
Delta State University Teaching Hospital Oghara	2
Federal medical center Abeokuta	3
Federal Medical Center Azare	2
Federal medical center birnin kebbi	1
Federal medical center Jalingo	3
federal medical Center Umuahia	2
Federal medical center, Jabi Abuja	1
Federal Medical Center, Nguru	2
Federal Medical Centre Birnin-Kebbi	1
Federal Medical Centre Owo Ondo State	2
Federal Medical Centre Yenagoa	1
Federal Medical Centre, Bida	1
Federal Medical Centre, Bida	1
Federal Medical centre, Gusau	2
Federal Teaching Hospital Ido-Ekiti	2
Federal Teaching Hospital Lokoja	1
Federal Teaching Hospital, Gombe	2
FEDERAL TEACHING HOSPITAL, OWERRI	1
Federal Teaching Hospital, Owerri, Imo State	1
FTH KATSINA	2
Irrua specialist hospital	1
Jos University Teaching Hospital	2
Kogi state specialist Hospital Lokoja	2
Lagos University Teaching Hospital Idi Araba Lagos	5
LASUTH (Lagos STATE University Teaching Hospital)	1
MAUTH YOLA	2
National Hospital	1
National orthopedic hospital kano	1
Nnamdi Azikiwe university teaching hospital Nnewi Anambra state	3
Rasheed shekoni federal University teaching hospital dutse	2
Specialist Hospital Sokoto	1
The Univeristy of Abuja teaching hospital	1
UDUTH	1
Uniosun Teaching Hospital, Osogbo, Osun State	2
UNIVERSITY COLLEGE HOSPITAL, IBADAN	2
University of Benin Teaching Hospital	1
University of Calabar Teaching Hospital	3
University of Calabar Teaching Hospital Calabar	1
University of Ilorin teaching hospital	2
University of Maiduguri Teaching Hospital	2
University of Nigeria Teaching Hospital	1
University of Port Harcourt teaching hospital	3
University of Uyo Teaching Hospital, Akwa Ibom State	2

intersex
NIGERIA
CENTER FOR HEALTHCARE DEVELOPMENT & YOUTH EMPOWERMENT

This document was developed and published by Intersex Nigeria

website: intersexnigeria.org
email: info@intersexnigeria.org